Foundations NRSG 231 · Midterm Study Guide

Midterm: Wed Oct 21, 2026, in person on Examplify (proctored). The syllabus says "Module 1-5," but Week 5 has no new reading.

Covers: Ch. 25 Asepsis & Infection Control · Ch. 34 Activity · Ch. 26 Vital Signs (Pt 1 & 2) · Ch. 8 Communication Pt 1 · Weekly lab skills.

How it's built: her lecture slides come first [Slide N], then the assigned Taylor 10th ed. pages [Taylor p. N], then NCLEX test traps. Taylor page numbers are estimated from the ebook layout and can be off by about 1–2 pages. "⚠ Conflict" marks spots where her slides and Taylor differ; go with her slides.

Companion quiz: excellence-school-calendar.vercel.app/quizzes/foundations-midterm-mastery/

Assigned reading [Syllabus]: Ch. 25 pp. 663–684, Box 25-2, 25-4 · Ch. 34 Table 34-3 (adult/older adult), 34-6, Fig. 34-1, 34-2, Box 34-1 · Ch. 26 pp. 716–742, Box 26-1, Table 26-1 (adult), Box 26-3/4/5/6, Table 26-7, 26-9 · Ch. 8 pp. 172–180, 187–201, Box 8-4, 8-6.

Core tables (memorize these first)

Adult vital signs: normal ranges

Vital sign	Her slides	Taylor
Temperature	Tissues work best 36–37.5 °C (98.6 °F) [Slide 5]. Fever >40 °C (104 °F) dangerous; ≥41 °C (106 °F) = hyperpyrexia, a medical emergency [Slide 7]	Adult 96.4–99.5 °F / 35.8–37.5 °C [Taylor Table 26-1, p. 718]. Range by route 96.7–100.5 °F (35.9–38 °C) [Taylor p. 718]
Pulse	60–100, average 80 bpm. >100 tachycardia, <60 bradycardia [Slide 15]	60–100 [Taylor Table 26-1]
Respirations	12–20 breaths/min; ↑ tachypnea, ↓ bradypnea [Slide 17]	12–20 [Taylor Table 26-1]
Blood pressure	Normal <120/80. Hypotension <90 systolic or <60 diastolic. Elevated 120–129 systolic. Stage 1 130–139 / 80–89. Stage 2 ≥140 / ≥90. Crisis >180 [Slide 20]	Same categories: elevated = 120–129 **and** diastolic <80 [Taylor Table 26-7]
SpO_2	Pulse oximetry is a non-invasive estimate of arterial O_2 saturation [Slide 23]	Her slides and the assigned pages give no SpO_2 cutoff. Many references use about 95–100% as normal, so check with her.

⚠ Conflict: Slide 20 writes "Elevated: systolic 120–129 and diastolic **of 80**." Taylor and the AHA categories say diastolic **below** 80; 80–89 is already Stage 1. Learn: elevated = 120–129 / <80.

BP errors [Slide 21]: cuff too wide = false LOW; cuff too narrow or too loose = false HIGH; arm above heart = false LOW (below heart = false high). Cuff width covers 40% of arm circumference;

bladder covers 2/3 of the arm. Size the cuff by arm circumference, not by age. No BP on an arm with a PICC, an AV fistula, or on the mastectomy side.

Chain of infection: six links [Slide 3–4; Taylor Fig. 25-1, p. 665]

#	Link	How the nurse breaks it
1	Infectious agent	Early identification, cleaning, disinfection, sterilization
2	Reservoir	Hygiene, clean/dry linens and dressings, environmental cleaning, empty drainage containers
3	Portal of exit	Cover coughs and sneezes, contain drainage, PPE, dispose of soiled items properly
4	Mode (means) of transmission	**Hand hygiene** (the single most effective step), gloves, isolation precautions, don't shake linens
5	Portal of entry	Aseptic technique, keep skin intact, sterile catheters and dressings
6	Susceptible host	Nutrition, rest, immunizations, protective precautions for immunocompromised patients

Isolation precautions [Slides 26–31]

Type	Room	PPE	Examples
Standard	Any	Based on exposure: all blood, body fluids, secretions and excretions **except sweat**, non-intact skin, mucous membranes	Every patient
Contact	Private	Gown + gloves; dedicated equipment	MDROs (MRSA, VRE), C. diff (soap and water, not gel)
Droplet	Private, door may stay open	Surgical mask (+ gloves)	Influenza, pertussis
Airborne	Private, negative pressure, door closed	N95 respirator; patient wears a surgical mask for transport	TB, measles, varicella (chickenpox)
Protective (reverse/neutropenic)	Private (positive-pressure protective environment for the most severe cases)	Per facility protocol; healthy staff; no plants/flowers or standing water; restrict sick visitors [Taylor p. 689]	Severely immunocompromised, cancer treatment, transplant

PPE order [Taylor Skill 25-2, pp. 695–700; lab skill]

Donning (put ON)	Doffing (take OFF), method A
0. Hand hygiene 1. **Gown** (opening in back; tie neck and waist) 2. **Mask/respirator** (fit check if N95) 3. **Goggles/face shield** 4. **Gloves** (pulled over the gown cuffs)	1. **Gloves** (glove-in-glove) 2. **Goggles/face shield** (by headband or earpieces) 3. **Gown** (touch only the inside, roll inside out) 4. **Mask/respirator** (by the ties or elastic; N95 removed **after** leaving the room) 5. Hand hygiene

Memory: on goes **G-M-G-G** (gown, mask, goggles, gloves); off goes in alphabetical order, **G-G-G-M** (gloves, goggles, gown, mask). The dirtiest item comes off first and the mask comes off last.

Therapeutic vs non-therapeutic communication (quick table, full detail in the Communication section)

Therapeutic ✔	**Non-therapeutic ✘**
Active listening, silence, open-ended questions ("Tell me more about…")	Closed or excessive questioning, "why" questions (sound accusing)
Restating, paraphrasing, reflecting feelings ("You seem worried…")	False reassurance ("Everything will be fine")
Clarifying, focusing, summarizing	Giving advice / your own opinion ("If I were you…")
Sharing observations, offering self ("I'll sit with you")	Changing the subject, minimizing feelings, clichés
Validating, giving information	Approving/disapproving, judging, defending, arguing, stereotyping

Module 1 · Ch. 25 Asepsis & Infection Control

Infection & the infection cycle [Slide 1] [Slide 2] [Slide 3] [Slide 4]

- **Infection** = a disease state that results from the presence of **pathogens in or on the body**; a **six-part process** must occur for an infection to develop [Slide 3].
- **Six links (in order)**: 1) Infectious agent → 2) Reservoir → 3) Portal of exit → 4) Means of transmission → 5) Portal of entry → 6) Susceptible host [Slide 3].
- Goal of every nursing intervention: **"Break the Chain"** — interrupt one link and the cycle stops [Slide 4].
- **Learning objectives to know**: explain the infection cycle; list stages of infection; identify at-risk patients; describe nursing interventions that break the cycle; identify when hand hygiene is indicated; identify MDROs in hospitalized patients; implement CDC standard & transmission-based precautions; use medical and surgical asepsis [Slide 2].

The 6 links and how a nurse breaks each one

Link	What it is [Slides]	Nurse breaks it by
1. Infectious agent	Pathogenic microorganism [Slide 5]	Clean/disinfect equipment, sterile technique, aseptic "clean-to-dirty" care; treat/eradicate where prescribed [Slide 19] [Slide 29]
2. Reservoir	Where the organism lives & multiplies [Slide 6]	Hand hygiene, environmental cleaning, cough etiquette, do not share equipment, no soiled linen on floor [Slide 19] [Slide 26] [Slide 29]
3. Portal of exit	Route the organism leaves the body [Slide 11]	Gloves/gown for drainage & body fluids, contain dressings, dispose of contaminated items, respiratory hygiene/cough etiquette [Slide 19] [Slide 25] [Slide 26]
4. Means of transmission	Contact, droplet, airborne [Slide 8] [Slide 9]	Standard + transmission-based precautions, correct PPE donning/doffing, private room/negative pressure, hand hygiene [Slide 26]–[Slide 31]
5. Portal of entry	Where the organism enters the new host [Slide 7]	Sterile technique for invasive devices (foley, central line), protect skin integrity, avoid splashing contaminated liquids [Slide 19]
6. Susceptible host	A host whose defenses are overcome [Slide 12]	Support nutrition/fluids/rest, immunization status, reduce invasive device days, reduce stress/fatigue, monitor for infection [Slide 12] [Taylor p. 673]

Link 1 — Infectious agents [Slide 5]

- **Bacteria**: E. coli, staphylococcus, streptococcus [Slide 5].
- **Viruses**: HIV, common cold, influenza, COVID-19, hepatitis A, B, C [Slide 5].
- **Fungi**: athlete's foot, ringworm, yeast infections [Slide 5].
- **Parasites**: malaria occurs from mosquitoes infected with a parasite [Slide 5].

Link 2 — Potential reservoirs [Slide 6]

- People, animals, soil, food, water, milk, **objects** (inanimate objects = **fomites**) [Slide 6] [Slide 8].

- Nursing implication: equipment used repeatedly (e.g., stethoscope) can act as a **fomite** [Slide 10].

Link 5 — Portals of entry (humans) [Slide 7]

- Respiratory tract, gastrointestinal (GI) tract, genitourinary (GU) tract, **skin breakdown**, blood & tissues [Slide 7].
- Protect these portals during all patient contact [Slide 10].

Link 4 — Modes of transmission [Slide 8] [Slide 9]

- **Contact transmission** [Slide 8]:
  - **Direct contact**: close proximity — touching, kissing, sexual intercourse [Slide 8].
  - **Indirect contact**: personal contact with a **vector** (living creature that transmits a pathogen to humans) or a **fomite** (inanimate object that harbors a pathogen) [Slide 8].
- **Droplet transmission**: organisms >5 µm; droplets are heavy enough to land on surfaces [Slide 9].
- **Airborne transmission**: organisms <5 µm; airborne particles remain suspended in the air [Slide 9].

⚠ Conflict: Slide 9 states both droplet (>5 µm) and airborne (<5 µm) organisms "attach to dust particles" [Slide 9], whereas Taylor attributes dust-particle spread to the **airborne** route and defines **droplet** spread as large-particle droplets from coughing/sneezing/talking that travel <3 ft (up to 6 ft) [Taylor p. 668] [Taylor p. 684]. **Go with the slides for her exam.**

Link 3 — Portals of exit (humans) [Slide 11]

- Respiratory tract, GI tract, GU tract, skin breakdown, **blood & drainage** [Slide 11].
- Exit routes usually match entry routes of the next host [Slide 7] [Slide 11].

Link 6 — Susceptible host [Slide 12] [Slide 13] [Slide 14]

- Multiple wounds / poor skin integrity = **multiple portals of entry** [Slide 12].
- **pH** of GI and GU tracts and skin protects against microbial invasion [Slide 12].
- **WBC count**; age, sex, heredity (which age groups are most susceptible?) [Slide 12].
- **Immunity**: natural vs acquired vs passive [Slide 12].
- Level of fatigue; nutritional status/general health; pre-existing illness; certain medications [Slide 12].
- Stress level; use of **invasive or indwelling medical devices** [Slide 12].
- Case study factors that create a susceptible host: open laceration, active infection (COVID-19), dehydration, IV device, fatigue/overwork, incomplete immunization status [Slide 13].
- Slide 14 is image/figure-only (no text) [Slide 14].

What makes an organism thrive? [Slide 15]

- Quantity of organism; **virulence**; competence of the patient's immune system; how long organism and patient were in contact; **type of contact** [Slide 15].

Targeting a specific link (means of transmission) [Slide 16]

- Practice item covers identifying the nursing action that breaks the chain at the **"mode of transmission"** — the correct concept is a **barrier (PPE/gloves)**, not antibiotics, nutrition, or monitoring [Slide 16].

Stages of infection [Slide 17] [Slide 18]

- **1. Incubation period**: pathogen invades → first symptoms; organism growing/multiplying. Examples: common cold 1–2 days; tetanus 2–21 days [Slide 17].
- **2. Prodromal stage**: hours to days; vague, nonspecific symptoms (fatigue, malaise, low-grade fever) [Slide 17].
- **3. Full stage of illness**: infection-specific signs and symptoms [Slide 17].
- **4. Convalescent period**: illness resolved, period of recovery, return to baseline health; there may be a **temporary or permanent** change in the patient's previous health [Slide 17].
- A person can **continuously cycle** through the same infectious process — e.g., HSV [Slide 17].
- Knowledge check topic: interpreting vague fatigue + low-grade fever by stage [Slide 18].

Medical vs surgical asepsis [Slide 19] [Slide 20]

	Medical asepsis	**Surgical asepsis**
Also called	**"Clean technique"** [Slide 19]	**"Sterile technique"** [Slide 19]
Goal	**Reduce** the number of pathogens; reduce transfer [Slide 19]	**Eliminate** pathogens; create and maintain a **pathogen-free environment** [Slide 19]
Techniques	Hand hygiene; wearing gloves; do not place dirty/soiled linen on the floor; avoid raising dust; **"clean to dirty"** technique; dispose of dirty linens directly into hamper; pour contaminated liquids directly into the drain or **"hopper"** to prevent splashing [Slide 19]	Sterile gloves, mask, hair net, gowns, etc. [Slide 19]
Used for	Routine care of all patients (everyday nursing activities) [Slide 19]	Procedures such as **foley catheter insertion** and **central line placement/dressing changes** [Slide 19]

Hand hygiene rules [Slide 21] [Slide 24]

- **WHO "Five Moments for Hand Hygiene"** [Slide 21]:
    1. Before touching a patient
    2. Before a clean or aseptic procedure
    3. After body fluid exposure risk
    4. After touching a patient
    5. After touching patient surroundings
- Hand hygiene is the **single best/most effective** way nurses prevent infection [Slide 32].
- Knowledge check topics: duration of handwashing with soap and water vs hand sanitizer, which is more effective, and when sanitizer is **not** appropriate [Slide 24] — see Taylor rules below.

Health care–associated infections (HAIs) [Slide 22]

- Also known as **hospital-acquired / nosocomial** infections [Slide 22].
- **CAUTI** – catheter-associated urinary tract infection [Slide 22].
- **SSI** – surgical site infection [Slide 22].
- **CLABSI** – central-line associated bloodstream infection [Slide 22].
- **VAP** – ventilator-associated pneumonia [Slide 22].

Multidrug-resistant organisms (MDROs) [Slide 23] [Slide 24]

- **VRE** (vancomycin-resistant enterococci) — found in feces, urine, blood [Slide 23].
- **MRSA** (methicillin-resistant *Staphylococcus aureus*) — nares, skin, GI tract [Slide 23].
- **CRE** (carbapenem-resistant Enterobacteriaceae) [Slide 23].
- **C. diff** (*Clostridium difficile*) — feces [Slide 23].
- Knowledge check topics: signs/symptoms of infection and what an infection + medical device suggests [Slide 24].

Personal protective equipment (PPE) [Slide 25]

- **Gloves**, **gown**, **masks**, **protective eye-wear** [Slide 25].

Standard precautions [Slide 26]

- Used in the care of **ALL hospitalized patients regardless of diagnosis or possible infection status** [Slide 26].
- Apply to blood, **all body fluids, secretions, and excretions except sweat** (whether blood is present or visible), **non-intact skin**, and **mucous membranes** [Slide 26].
- **New additions**: respiratory hygiene/cough etiquette; safe injection practices; use a mask when performing **high-risk prolonged procedures involving spinal canal punctures** [Slide 26].

Transmission-based precautions [Slide 27]

- Used **in addition to** standard precautions for patients with suspected infection with pathogens transmitted by airborne, droplet, or contact routes [Slide 27].
- **Don PPE when entering** the room; **doff PPE when leaving** [Slide 27].
- A disease may have **multiple routes of transmission** (airborne, droplet, contact) [Slide 27].

Standard vs transmission-based precautions at a glance

Precaution	Use for	Room / environment	PPE & key actions
Standard [Slide 26]	All patients, every time, regardless of diagnosis [Slide 26]	Any room; environmental cleaning; review room assignment (e.g., private room for incontinent patient) [Slide 26]	Hand hygiene; gloves for blood/body fluids, mucous membranes, non-intact skin; gown/mask/eye protection for splash or spray risk; cough etiquette; safe injection practices [Slide 25] [Slide 26]
Contact [Slide 29]	Infected or **colonized by an MDRO** [Slide 29]	**Private room**; clean all equipment — **do not share** [Slide 29]	**Gown and gloves**; **change gloves after contacting infective material**; doff before leaving room [Slide 27] [Slide 29]
Droplet [Slide 28]	Infection spread by **large-particle droplets** (>5 µm) [Slide 9] [Slide 28]	**Private room; door may stay open** [Slide 28]	**Surgical mask and gloves** [Slide 28]; surgical mask on patient for transport [Slide 27]
Airborne [Slide 30]	Infections that spread through the **air** (<5 µm particles) [Slide 9] [Slide 30]	**Private room; negative air pressure; door**	**Respirator mask (N95-type)** for staff; **surgical**

		closed; patient remains in room [Slide 30]	**mask on patient for transport** [Slide 30]
Protective / reverse (neutropenic) [Slide 31]	**Severely immunocompromised & immunosuppressed** patients — cancer treatment, transplants, **WBC < 500 cells/mm³ (µL)** [Slide 31]	Protective environment for the vulnerable patient (reverse isolation) [Slide 31]	Protect the **patient** from staff/visitor flora — meticulous hand hygiene and barriers before contact [Slide 21] [Slide 31]

Wrap-up points [Slide 32] [Slide 33] [Slide 34]

- Best nursing prevention of infection = **hand hygiene** + consistent aseptic technique and precautions [Slide 32].

From the assigned reading (Taylor)

- **Colonization vs infection**: normal flora are nonpathogenic residents; **opportunists** (e.g., E. coli from gut → UTI; C. diff) cause disease when defenses fail. C. diff present with no clinical signs = **colonization**; with manifestations = **infection** [Taylor p. 667].
- **Carriers**: asymptomatic reservoirs who can still transmit (e.g., HIV-positive person with no symptoms; maternal-fetal transmission) [Taylor p. 667].
- **Bacteria categories**: shape (cocci, bacilli, spirochetes); **gram-positive** (thick wall, stain violet) vs **gram-negative** (do not stain) — drives antibiotic choice; **aerobic** (need O_2) vs **anaerobic** [Taylor p. 664].
- **Viruses**: smallest, need electron microscope; **antibiotics have no effect**; antivirals (e.g., influenza, HSV-1 in the **prodromal** stage) shorten illness; HIV treated with ART [Taylor p. 664].
- **Fungi/parasites**: athlete's foot, ringworm, yeast (antifungals; many resistant); malaria via infected mosquito [Taylor p. 665].
- **Endemic vs pandemic**: endemic = predictable in one region/population; pandemic = global outbreak of a novel virus (COVID-19) [Taylor p. 666].
- **Table 25-1 examples**: Lyme disease (ticks, indirect/vector contact); E. coli (feces/food/water, direct/indirect); HBV (blood/body fluids, direct); HIV (blood, semen, vaginal secretions, breast milk — direct contact); TB (sputum — **airborne**); Salmonella (intestinal tracts — direct/indirect); *S. aureus* (skin, mouth, nose, throat — direct/indirect; abscesses, endocarditis, osteomyelitis, bacteremia) [Taylor Table 25-1, p. 669].
- **Defense mechanisms**: first line = skin, mucous membranes, normal flora; then inflammatory and immune responses. Hallmarks of acute inflammation = **redness, heat, swelling, pain, loss of function**; vascular then cellular phase; neutrophils phagocytize; exudate = serous, sanguineous, or purulent; repair by regeneration or scar tissue [Taylor p. 670].
- **Immune response**: **antigen–antibody (humoral) immunity** plus **cell-mediated immunity** (lymphocytes) [Taylor p. 670] [Taylor p. 671].
- **Localized vs systemic infection signs**: localized = redness, swelling, warmth, pain/tenderness, loss of function; systemic = fever with increased pulse & respirations, lethargy, lymph node tenderness/enlargement, anorexia [Taylor p. 672].
- **Older adults**: atypical infection presentation — confusion, disorientation, lethargy, anorexia, delayed fever response, falls, incontinence, failure to thrive; age risks include pulmonary, UTI, and skin infections (decreased cough reflex, incomplete bladder emptying, thinning epidermis) [Taylor Focus on the Older Adult, pp. 671–672].

- **Box 25-2 medical asepsis basics**: carry soiled linens away from clothing; nothing on the floor; tissue for coughing/sneezing; move equipment away when dusting/brushing; avoid raising dust (damp cloth; don't shake linens); **clean least soiled → most soiled**; wrap moist items in waterproof bags; pour liquids directly into drain; sterilize contaminated equipment; short clean nails, no rings with grooves, hair restrained; follow standard/transmission-based precautions [Box 25-2, p. 674].
- **Hand flora**: **transient** bacteria (loose, in grease/dirt, greatest under nails — easily removed by handwashing) vs **resident** bacteria (skin creases, cling tenaciously, need friction/brush, less susceptible to antiseptics) [Taylor Table 25-2, p. 676].
- **Nails**: no artificial nails for patient care (OR violation = citable); natural nails < 1/4 inch [Taylor p. 676].
- **Hand hygiene method & timing**: alcohol-based handrub (60%–95% alcohol) when hands are **not visibly soiled** — apply recommended amount (≈3 mL), cover all surfaces/interdigital areas/nails, rub **until completely dry (at least 20 seconds)**; wash with **antimicrobial soap and water for at least 20 seconds** when hands are visibly soiled or contaminated with blood/body fluids, and before eating and after the restroom; longer scrub if visibly soiled [Taylor p. 677, Guidelines for Nursing Care 25-1].
- **Alcohol rubs are ineffective against** *Cryptosporidium*, **norovirus, and C. diff (spores)** — use soap and water; contact precautions and gown/gloves until diarrhea stops [Taylor p. 677].
- **Gloves**: not a substitute for hand hygiene; single use; change between tasks and before moving contaminated → clean; warmth/moisture inside gloves promotes bacterial growth; hand hygiene before and after glove use; **double gloving** reduces perforation (71% in OR); do not touch charts, keyboards, phones, or leave the unit in gloves [Taylor p. 682].
- **Latex** (Box 25-3): irritant contact dermatitis (not an allergy), allergic contact dermatitis (type IV, 24–96 hr), true latex allergy (type I, anaphylaxis); ~10% of health care workers sensitized; powder binds latex protein and becomes airborne → move to powder-free gloves; medical alert bracelet; no cure, avoidance only [Box 25-3, pp. 682–684].
- **Masks/eyewear**: surgical mask filters expired air (large droplets >5 µm, droplets travel <3 ft, up to 6 ft); particulate respirator (N95) filters inspired air and particles ≤5 µm; **fit test required** and **seal check each use**; AII room for TB = negative pressure, 6–12 air changes/hour, HEPA or exhaust to outside; change mask before it becomes damp; goggles/face shield with side shields for splash risk (plain glasses not acceptable) [Taylor pp. 684–685].
- **HAI terms**: **exogenous** (organism from other people), **endogenous** (from the patient's own flora), **iatrogenic** (result of a treatment/diagnostic procedure); CMS non-reimbursement for preventable HACs; mandatory public HAI reporting in most states [Taylor p. 678] [Taylor p. 679].
- **Bundles** = 3–5 evidence-based practices implemented together; CAUTI risk rises 3%–7% per day the catheter stays in; VAP bundle includes HOB 30°–45°, regular oral care, changing ventilator circuit only as needed [Taylor pp. 678–679].
- **MDROs**: MRSA (HA-MRSA: IV vancomycin first line; CA-MRSA: often incision & drainage, culture before antibiotics); VRE (feces/urine/blood; linezolid or daptomycin); CRE (40%–50% mortality; contact precautions); CRAB (clean/disinfect environment); C. diff (watery diarrhea, fever, cramping; sporicidal terminal cleaning, soap and water, antibiotic stewardship) [Taylor pp. 679–681].
- **Disinfection vs sterilization**: disinfection destroys pathogens **except spores**; sterilization destroys **all** microorganisms **including spores**. Sterilize items entering a sterile body part. Methods (Table 25-3): steam/autlave, boiling ≥10 min, dry heat 350°F 2+ hr, radiation, ethylene oxide gas (toxic), chemical solutions (bleach 1:100 inactivates HIV; betadine, alcohol). Selection

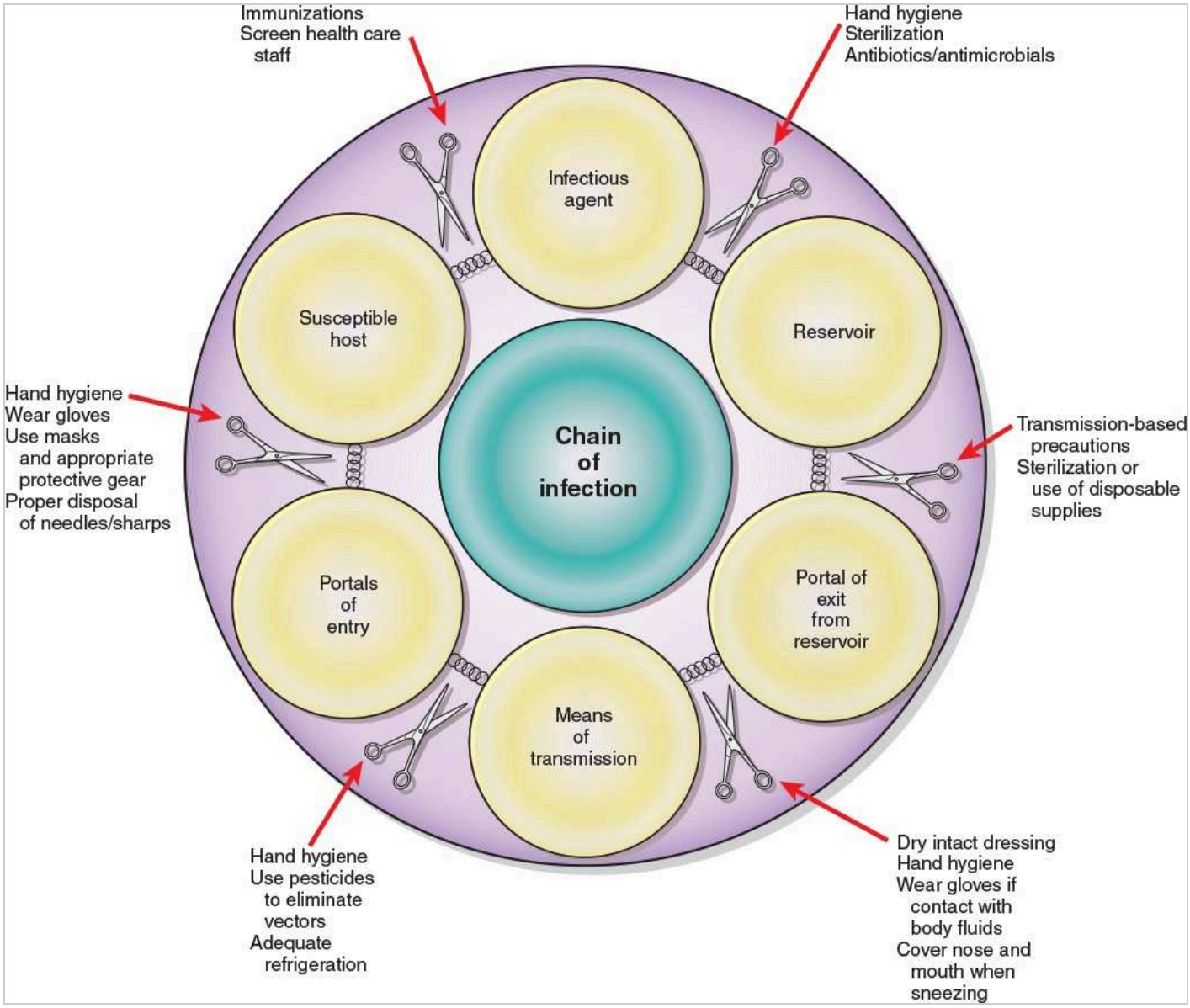
factors: organism, number, equipment type, intended use, available means, **time** [Taylor pp. 681–683, Table 25-3].

- **Supplies/waste**: never reuse a contaminated item on another patient; double bag if outer bag soiled; body-fluid spills cleaned immediately with appropriate germicide; red **BIOHAZARD** bag for liquid/semiliquid OPIM and caked dried blood; paper trays/plastic utensils do **not** prevent transmission (commercial dishwashers suffice) [Taylor p. 685].
- **Box 25-4 CDC highlights not in the slides**: no recapping needles — one-handed scoop or recapping device; safety-engineered devices; puncture-resistant sharps containers; safe injection practices (single-dose vials, new needle/syringe per injection); mask for spinal/epidural procedures; droplet visitors kept 3 ft from infected person; transport droplet/airborne patients only when necessary with surgical mask on the patient [Box 25-4, pp. 687–688].
- **Occupational exposure**: OSHA Bloodborne Pathogens Standard (1991) and Needlestick Safety and Prevention Act (2000) — safer devices, worker involvement in device selection, sharps injury log, education; sharps injuries in ~40% of cases occur in surgery [Taylor p. 688].
- **Isolation** = a protective procedure limiting spread among patients, personnel, and visitors; transmission-based types may be combined but always add to standard precautions [Taylor p. 685] [Taylor p. 686].

NCLEX test traps

- **Trap: "Hand sanitizer is always the best method."** Alcohol rubs are first-line when hands are **not visibly soiled**, but soap and water are required for visibly soiled hands, blood/body fluid contamination, before eating, after the restroom, and for **C. diff, norovirus, Cryptosporidium** [Slide 24] [Taylor p. 677].
- **Trap: swapping droplet and airborne.** Droplet = **>5 µm**, **surgical mask**, private room, **door may stay open**; airborne = **<5 µm**, **respirator (N95)**, **negative pressure**, **door closed**, patient stays in room [Slide 28] [Slide 30].
- **Trap: "Standard precautions apply only to patients in isolation or with a known infection."** They apply to **every patient, every time**, regardless of diagnosis [Slide 26].
- **Trap: listing sweat as a body fluid requiring precautions.** Standard precautions cover all body fluids **except sweat** [Slide 26].
- **Trap: "Gloves replace hand hygiene" or reusing gloves between patients/tasks.** Gloves are single-use, changed between tasks and before moving contaminated → clean, and hand hygiene is performed before and after glove removal [Slide 29] [Taylor p. 682].
- **Trap: choosing antibiotics, nutrition, or vital-sign monitoring when asked what breaks the chain at the "means of transmission."** The answer is a **barrier/PPE or isolation technique** [Slide 16] [Slide 27].
- **Trap: mixing up medical ("clean," *reduce* pathogens) vs surgical ("sterile," *eliminate*) asepsis.** Sterile technique is mandatory for foley catheter insertion and central line placement/dressing changes [Slide 19].
- **Trap: labeling vague fatigue/malaise/low-grade fever as "incubation."** Incubation = **no symptoms yet**; vague early symptoms = **prodromal**; specific symptoms = **full illness** [Slide 17] [Taylor p. 670].
- **Trap: forgetting protective (reverse/neutropenic) precautions.** They protect the **immunocompromised** host — chemotherapy, transplants, **WBC < 500 cells/mm³** — not the community [Slide 31].
- **Trap: thinking a fomite is only a piece of trash or that stethoscopes are safe.** Any inanimate object (stethoscope, BP cuff, phone, keyboard) can harbor pathogens; clean/disinfect

between patients and never share equipment [Slide 8] [Slide 29] [Taylor p. 668].



Taylor Fig. 25-1: The chain of infection (p. 665)

Her slides: pictures



Ch. 25 slide 1



Ch. 25 slide 2

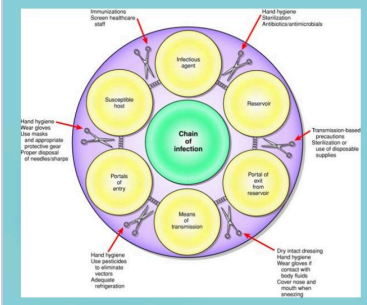
Infection

A disease state that results from the presence of pathogens in or on the body. A six-part process must occur in order for an infection to occur:

- Infectious agent
- Reservoir
- Portal of exit
- Means of transmission
- Portal of entry
- Susceptible host



Ch. 25 slide 3



The Infection Cycle: "Break the Chain"

Ch. 25 slide 4

Infectious agents

Bacteria
E.coli, staphylococcus, streptococcus

Virus
HIV, common cold, influenza, COVID-19, hepatitis A,B,C

Fungi
Athlete's foot, ring worm, yeast infections

Parasites
Malaria occurs from mosquitos infected with a parasite

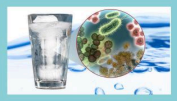


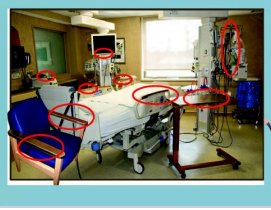



Ch. 25 slide 5

Potential Reservoirs

People
Animals
Soil
Food
Water
Milk
Objects






Ch. 25 slide 6

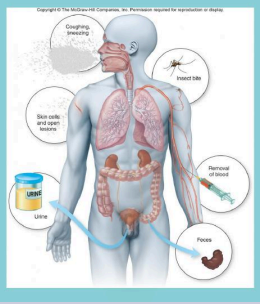
Portals of Entry for Humans

- Respiratory
- Gastrointestinal
- Genitourinary Tracts
- Skin breakdown
- Blood & Tissues









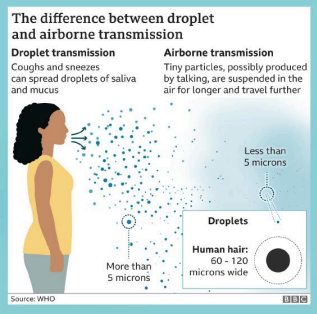
Ch. 25 slide 7

Mode of Transmission

Contact Transmission
- Direct contact: close proximity
  - Touching, kissing, sexual intercourse
- Indirect contact
  - Personal contact with
    - A vector- living creature that transmits a pathogen to humans
    - A fomite - an inanimate object that harbors a pathogen



Ch. 25 slide 8



Mode of Transmission

Droplet transmission
- Organisms greater than 5um (micrometer) attach to dust particles
- Droplets are heavy enough to land on surfaces

Airborne transmission
- Organisms less than 5um (micrometer) attach to dust particles
- Airborne particles remain in the air

Ch. 25 slide 9

Knowledge Check



What object do nurses use that could potentially act as a fomite?

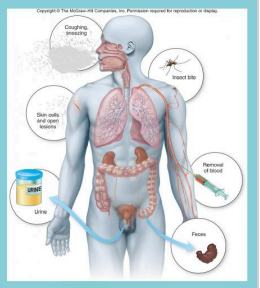
What objects do we come into contact with that act as fomites?

Which portal of entries do nurses need to protect while in contact with patients?

Ch. 25 slide 10

Portals of Exit for Humans

- Respiratory
- Gastrointestinal
- Genitourinary Tracts
- Skin breakdown
- Blood & Drainage



Ch. 25 slide 11

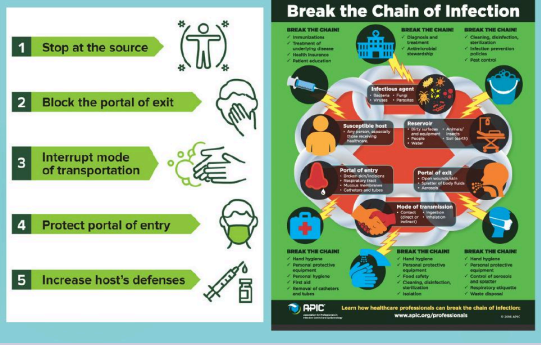
Case Study



Jimin is a 27 year old nursing student. He is a biological male of Asian descent. He has received his childhood vaccinations but is unvaccinated for COVID. He is in his 4th quarter and is also working part-time as a pop star. During a rigorous dance number, he fell on stage and sustained a laceration to his left leg. Despite his injury, he continued to go to school as well as perform. During this time, he became hospitalized for COVID-19 and dehydration. Due to his need for IV fluid, he has an IV placed into his left arm.

What factors make this patient a susceptible host?

Ch. 25 slide 13



Ch. 25 slide 14

What Makes an Organism thrive?

- Quantity of Organism
- Virulence
- Patient's immune system
- How long the organism + patient were in contact
- What type of contact they had



Ch. 25 slide 15

Practice Question

A patient with a wound infection is placed in isolation to prevent the spread of infection. Which action by the nurse targets breaking the chain of infection at the "mode of transmission"?

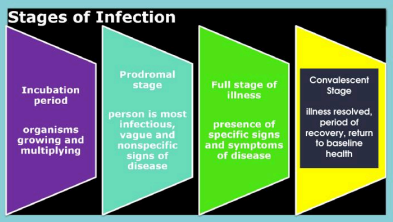
A. Administering antibiotics as prescribed
B. Wearing gloves while providing wound care
C. Ensuring the patient receives proper nutrition
D. Monitoring the patient's temperature regularly

Ch. 25 slide 16

Stages of Infection

Incubation Period
  Common cold: 1-2 days
  Tetanus: 2-21 days
Prodromal Stage
  Hours to days
Full Stage of Illness
Convalescent Period
  Maybe a temporary or permanent change in the patient's previous health

Can a person continuously cycle through the same infectious process?
  HSV



Ch. 25 slide 17

Knowledge Check



1. A school nurse is performing an assessment of a student who states, "I'm too tired to keep my head up in class." The student has a low-grade fever. The nurse would interpret these findings as indicating which stage of infection?
   a. Incubation period
   b. Prodromal Stage
   c. Full Stage of Illness
   d. Convalescent period

Ch. 25 slide 18

Asepsis- Methods to Prevent Infection

Medical- Reduce # of Pathogens

"Clean Technique"
Hand Hygiene
Wearing gloves
Do not place dirty, soiled linen on the floor
Avoid raising dust
Practice "clean to dirty" technique
Dispose of dirty linens directly into hamper
Pour contaminated liquids directly into the drain or into the "hopper" to prevent splashing

Surgical- Eliminate # of Pathogens

"Sterile Technique"
Create and maintain pathogen free environment
Required for procedures such as foley catheters and central line placement/dressing changes
Sterile gloves, mask, hair net, gowns etc

Ch. 25 slide 19

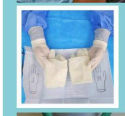
Asepsis- Methods to Prevent Infection





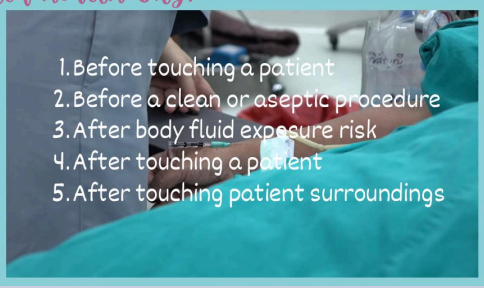

Medical versus Surgical Asepsis

Medical Asepsis	Surgical Asepsis
Clean technique used	Sterile technique used
Controls microorganisms	Absence of microorganisms
Basic hand hygiene used	Surgical scrub performed
Clean equipment and supplies	Sterile equipment and supplies
Clean field	Sterile field


Ch. 25 slide 20

Five Moments for Hand Hygiene (World health Org)



Ch. 25 slide 21

Healthcare Associated Infections (HAIs)

- Also know as hospital acquired/ nosocomial infections
  - Catheter Associated Urinary Tract Infections (CAUTI)
  - Surgical Site Infection (SSI)
  - Central-Line Associated Bloodstream Infection (CLABSI)
  - Ventilator-Associated Pneumonia (VAP)






Ch. 25 slide 22

Multidrug Resistant Organisms

- Vancomycin Resistant Enterococci (VRE)
  - Feces, urine, blood
- Methicillin-Resistant Staphylococcus Aureus (MRSA)
  - Found in the nares, on the skin, or in the GI tract
- Carbapenem-Resistant Enterobacteriaceae (CRE)
- Clostridium Difficile (C. Diff)
  - feces



Ch. 25 slide 23

Knowledge Check



1. What are some signs/symptoms of infections?
2. If a patient is displaying symptoms of infection and there is a medical device present, what can we determine?
3. How long do we wash hands with soap and water?
4. Hand sanitizer?
5. Which method is more effective?
6. When is hand sanitizer not appropriate?

Ch. 25 slide 24

Personal Protective Equipment

Gloves
Gown
Masks
Protective Eye-Wear










Ch. 25 slide 25

Transmission Based Precautions

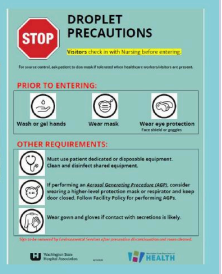
- Used in addition to standard precautions for patients in hospitals with suspected infection with pathogens that can be transmitted by airborne, droplet, or contact routes.
- Proper Donning of personal protective equipment (PPE) when entering the room of a patient on contact or droplet precautions.
- Proper Doffing of PPE when leaving the room
- These categories recognize that a disease may have multiple routes of transmission: airborne, droplet, contact.



Ch. 25 slide 27

Droplet

Infection spreads by large particle droplets
Private room
Door can stay open
Surgical mask and gloves

Ch. 25 slide 28

Contact

- Infected or colonized by a multi drug resistant organism (MDRO)
- Private room
- Gown and gloves
- Change gloves after contacting infective material
- Clean all equipment; do not share





Ch. 25 slide 29

Airborne

Infections that spread through the air
Private room
Negative air pressure
Door closed, pt remains in room
Respirator mask
Surgical mask on patient for transport





Ch. 25 slide 30

ReverseNeutropenic

- Recommended for severely Immunocompromised & Immunosuppressed
- Residents receiving any cancer treatment or transplants
- Residents with WBC below 500 cells/mm3 (µL)



Ch. 25 slide 31

Knowledge Check



What is the best way nurses can prevent infection?

Ch. 25 slide 32



Ch. 25 slide 34

Module 2 · Ch. 34 Activity & Safe Patient Handling

Instructor slides (primary — know these first)

- **Scope:** Body mechanics & activity, Chapter 34 **[Slide 1]**
- Transition/placeholder slide only (no testable content) **[Slide 2]**
- **Normal movement and alignment [Slide 3]**
  - **Body alignment / posture** = optimal musculoskeletal balance and operation; **no undue strain** on joints, muscles, tendons, ligaments.
  - **Balance** = center of gravity (COG) close to its **base of support**, and the object has a **wide base of support**.
  - Balance **increases** when feet are spread apart and hips/knees are flexed → **decreases musculoskeletal strain**.
- **Coordinated body movement** = muscles working together for purposeful movement; use the **major, stronger muscle groups [Slide 4]**
- **Postural reflexes** maintain body position and equilibrium during rest or movement; include sensory organs of the **inner ear, vision, and autonomic contractions [Slide 4]**
- **Factors affecting movement and alignment [Slide 5]**
  - **Developmental:** infant → toddler → child → adolescent → adult → older adult. Promote neuromuscular development; facilitate each patient's use of the body for self-care.
  - **Physical health — postural abnormalities (congenital/acquired):** hip dysplasia, torticollis, scoliosis.
  - **Physical health — bone formation/muscle development:** dwarfism, osteoporosis, muscular dystrophy.
  - **Physical health — joint/bone mobility:** arthritis, trauma.
  - **Physical health — CNS:** stroke, spinal cord injury.
  - **ADLs** = self-care activities maintaining quality of life: **eating, bathing, dressing, toileting**.
  - **Mental health: depression** affects activity level, posture, and energy to perform ADLs.
  - **Lifestyle:** diet; sedentary vs active; smoking.
  - **Culture and values:** cultures differ in emphasis on importance of movement and exercise.
- **Safe patient handling and mobility — ergonomics** = designing equipment and work tasks to conform to the capability of the worker; adjusts work environment/practices to **prevent injuries [Slide 6]**
- Use proper body movement in **all** everyday activities — including simple ones (moving a medication cart, pushing a bed, reaching to silence an alarm) — to build good habits while engaging the musculoskeletal system **[Slide 6]**
- **Application of ergonomics to prevent injury [Slide 7]**
  - Develop a habit of **erect posture with correct alignment**.
  - Use the **longest and strongest muscles of arms and legs** for maximum power; **back muscles are weaker** and prone to injury when used improperly.
  - **Engage core muscles** to stabilize pelvis and protect abdomen: **gluteal muscles downward, abdominal muscles upward**.
  - Work **as close to the object as possible** so COG stays near the object → burden borne by legs and arms, not the back.
  - **Face the direction of movement; do not twist the body.**

    - **Push rather than pull** when possible; keep arms close and push with the **whole body**.
    - **Broaden base of support**: feet shoulder-width apart, knees flexed.
- Image-only slide (no text content) **[Slide 8]**
- **Ensuring safe patient handling — 10 steps [Slide 9]**
    1. **Assess the patient** and ability to assist with planned movement: ability to understand instructions and cooperate; know medical diagnosis, health issues, movement capabilities; apply braces/devices needed; encourage the patient to assist as permitted to promote **independence**.
    2. **Ensure enough staff** are available and present to move the patient safely.
    3. **Assess the area** for clutter, accessibility, and availability of devices; remove obstacles/barriers.
    4. **Use a screening or assessment tool.**
    5. **Decide which equipment** to use.
    6. **Plan carefully and communicate** to care team members and the patient.
    7. **Administer prescribed analgesic** if the patient is in pain **prior to** movement.
    8. **Elevate the bed** as necessary.
    9. **Lock the wheels.**
    10. **Ensure patient safety during transfer:** avoid friction on the skin; support the body properly with good alignment and **avoid grabbing**; move in a **smooth, rhythmic motion**.
- **What leads to patient handling injuries [Slide 10]**
    - Uncoordinated lifts.
    - High exertion while in an awkward position.
    - **Manual lifting/transferring without assistive devices.**
    - Repetitive movements (lifting, transferring, repositioning).
    - Lifting while fatigued; standing for long periods.
    - Transferring patients from beds to gurneys or wheelchairs; repositioning patients in bed.
    - Transferring/repositioning patients with **cognitive impairments**, who are **dependent or obese**, or who are unable/unwilling to actively participate and promote their own movements.
- **Devices to aid in mobility [Slide 11]**: friction-reducing sheets; gait belts; standing assist; lateral assist device; powered full-body lifts.
- **Patient positioning** (topic lead-in) **[Slide 12]**
- **Skills lab activities — next week (Module Week 3) [Slide 13]**: turning the patient in bed; moving a patient up in bed; bed → gurney; bed → chair; ambulating a patient; dangling a patient.

From the assigned reading

Mechanics, leverage and balance (Figs 34-1, 34-2, 34-7)

- **Levers:** skeletal muscles produce movement by **pulling on bones**; bones act as **levers** and joints act as **fulcrums** (forearm lifting a weight) **[Fig 34-1, Taylor p. 1244]**
- **Balance:** most stable when the **line of gravity passes through a wide base of support**; less stable with a narrow base; **unstable** when the line of gravity falls **outside** the base **[Fig 34-2, Taylor p. 1245]**
- **Correct lifting:** long/strong muscles of arms and legs, object held so line of gravity falls within base of support. **Incorrect:** pull on back muscles and leaning puts line of gravity outside the base **[Fig 34-7, Taylor p. 1263]**

- Use the **weight of the body to push** an object by falling/rocking **forward** and to **pull** by falling/rocking **backward [Taylor p. 1263]**
- Surface should be **dry and smooth** to decrease friction — rough, wet or soiled surfaces increase friction and effort **[Taylor p. 1263]**
- Flex the knees, **"put on the internal girdle"** (engage core), and come down close to the object to be lifted; **break heavy loads into smaller loads**; take breaks to relax and recover **[Taylor p. 1264]**

Developmental variations — Table 34-3

Age and degree of neuromuscular development markedly influence body proportions, posture, body mass, movements, and reflexes **[Taylor p. 1246]**. For an immobile child, obtain the **history of activity level before the injury** and build an **age-appropriate** plan to maximize function **[Taylor p. 1247]**.

Level	Movement/development highlights	Assessment priorities	Interventions
Infant	Activity/alertness alternate with quiet and sleep; 3 mo raises chest/head prone; ~5 mo head control; 3–6 mo sit/head control; 6–9 mo sits steadily, rolls over, creeps, pulls to stand, improved hand–eye; 9–12 mo progressing to unassisted walking, picks up small objects	Milestones at 3–6, 6–9, 9–12 months	Encourage parents to examine baby; respond to concerns about minor variations; emphasize individual variation; account for prematurity **[Table 34-3, Taylor pp. 1246–1247]**
Toddler	15 mo walk unassisted; 18 mo run; 2 yr jump; 3 yr stack blocks, simple puzzles, dress self	Walking/running/jumping; small-muscle coordination (dress, wash hands, brush teeth); distinguish slow developers from developmental lags	Teach safe environment; enthusiastically praise mastery; set limits so toddler does not overextend **[Table 34-3, Taylor pp. 1247–1248]**
Child	Age 4: stairs, walk backward, hop on one foot; age 5: skip, jump rope, jump off several steps; manipulates writing materials	Developmental charts for gross/fine motor; activity level and types of physically exerting play	Attitudes about body and exercise are developed in this period; reinforce safety teaching at each stage **[Table 34-3, Taylor p. 1248]**
Adolescent	Growth spurt, secondary sex characteristics; fitness can mean boundless energy; inactivity may start lifelong unhealthy behavior	Activity/exercise type; safety of recreational choices; **screen for scoliosis**; muscle mass, tone, strength, joint mobility	Encourage exercise, limit sedentary hobbies; lifestyle counseling; caution about "pushing too hard" **[Table 34-3, Taylor p. 1248]**

Adult	Erect sitting/standing, balanced coordinated movement; **pregnancy shifts center of gravity**; activity levels vary greatly	Balance of activity vs rest; lifestyle factors/illnesses interfering with mobility or ADLs	Fitness counseling; clarify exercise misconceptions; design/monitor safe exercise programs; special care for mobility alterations **[Table 34-3, Taylor p. 1248]**

- **IADLs** (higher-level function) = housekeeping, meal preparation, management of finances, transportation — distinct from ADLs (eating, bathing, dressing, toileting) **[Taylor p. 1247]**

Safe handling details & dementia — Box 34-1

- Safe handling requires **assessment criteria, screening/algorithms, and proper use of equipment**; keep the patient in **good alignment** and protect from injury **[Taylor p. 1264]**
- Injury-prevention factors: **user training, device wear and tear, suitability of equipment, correct operation [Taylor p. 1264]**
- **Eliminate or reduce unnecessary tasks** to reduce injury risk and increase self-esteem and mobility; **patient engagement** during handling prevents adverse events **[Taylor p. 1264]**
- Use a **standardized screening tool** for consistency and appropriate SPHM equipment selection **[Taylor p. 1264]**

Safe handling of patients with dementia [Box 34-1, Taylor p. 1263] — communication problems and weakness make handling challenging:

- **Face the patient**; speak **slowly**; use **clear, short sentences**; call the patient **by name**; use a **calm, reassuring tone**; **allow time for a response**.
- Give **simple, one-step instructions**; repeat verbal cues/prompts as needed when thought processes are delayed.
- Determine if the patient has **receptive aphasia** (inability to understand) — this causes apparent noncompliance.
- **Phrase instructions positively:** say "Stand up" until the chair is positioned — not "Don't sit down" (the "don't" may not register; patient sits too early).
- Ask **one question at a time**, allow an answer, repeat if necessary.
- Allow focus on the task; **avoid correcting the process** unless it would be dangerous.
- **Incorporate established habits/customs** into desired activities (e.g., a patient used to evening baths may resist a morning shower; a patient may resist getting out of the "wrong side" of the bed).

Patient positions & complications to prevent — Table 34-6

- When the patient cannot self-reposition, use a **repositioning schedule**; frequency should be **evaluated, reevaluated, and tailored to the individual** — document rotation of positions **[Taylor p. 1269]**

Position	Definition / key points	Complication to prevent	Preventive action
Fowler's (head of bed **45–60°**) — promotes cardiac/respiratory function; position of choice for eating, conversation,	Flexion contracture of the neck	Head rests against mattress or small pillow only	

urinary/intestinal elimination. **High-Fowler's = 90°** (pillow on bedside table → maximal lung expansion); **low/semi-Fowler's = 30°**. Buttocks bear main weight; heels, sacrum, scapulae at risk for breakdown. **Avoid semi-Fowler's in patients at risk for skin integrity alterations** (shearing over sacrum)	Exaggerated curvature of the spine	Firm back support; elevation angle starts at the hips	
	Dislocation of the shoulder; wrist flexion contracture; edema of the hand	Forearms on pillows (no pull on shoulders); hand in natural alignment with forearm, slightly elevated relative to the elbow; hand–wrist splints for finger flexion/thumb abduction	
	Impaired lower extremity circulation & knee contracture; pressure on heels	Elevate knees briefly only; pillows under lower legs from below knees to ankles; avoid popliteal pressure and the knee gatch	
	External rotation of the hips	Trochanter roll	
	Footdrop	Feet supported in dorsiflexion; footboard or high-top sneakers	
	Source rows: **[Table 34-6, Taylor pp. 1270–1271]**		
Supine / dorsal recumbent (flat on back; head and shoulders slightly elevated with a pillow **unless contraindicated**, e.g., spinal anesthesia or spinal surgery)	Exaggerated spinal curvature; flexion of the hips	Firm supportive mattress; bed board if needed	
	Flexion contracture of the neck	Pillows under upper shoulders, neck, and head	
	Internal rotation of shoulders / extension of elbows (hunched shoulders); flexion of lumbar curvature	Pillows or arm supports under forearms, arms at sides, forearms pronated slightly; rolled towel or small pillow under lumbar curve if needed	
	Finger extension/thumb abduction; external rotation of the femurs; hyperextension of the knees	Hand–wrist splints if appropriate; sandbags or trochanter roll along hips and upper thighs; pillow under lower legs from below the knees **[Table 34-6, Taylor p. 1271]**	

Side-lying / lateral	Weight borne by **lateral aspect of lower scapula and lower ilium**; comfortable for sleep; relieves pressure on scapulae, sacrum, heels; support pillows required	See Table 34-6 for support pillows	**[Taylor p. 1270]**
Oblique / 30° side-lying	Hip of top leg flexed **30°** (not 90°), knee flexed **35°**, calf slightly behind midline; pillows behind back and under calf of top leg	Significantly **less pressure on trochanter and sacrococcygeal areas** vs. 90° side-lying	**[Fig 34-20, Taylor p. 1270]**
Sims'	Side-lying variation: lower arm behind patient, upper arm flexed at shoulder and elbow	Different pressure points — weight on **anterior humerus, clavicle, ilium**	**[Taylor p. 1270]**
Prone	On abdomen, head turned to side; shoulders/head/neck erect; hips extended; prevents hip/knee flexion contractures	**Lordosis** (contraindicated in **spinal problems**); **plantar flexion / footdrop** from gravity on feet	Position legs and feet carefully (see Table 34-6) **[Taylor p. 1270]**

- **VTE prophylaxis:** graduated compression stockings and pneumatic compression devices are **passive** interventions for venous stasis, thrombophlebitis, and venous thrombosis; may be used alone or with **pharmacologic** prophylaxis **[Taylor p. 1270]**

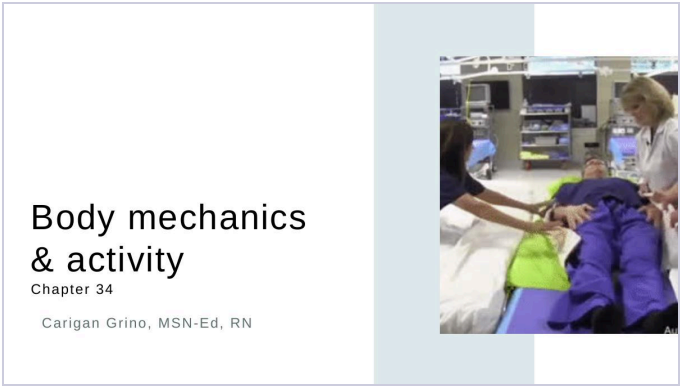
NCLEX test traps

- **Twisting the trunk** to move/reposition a patient is always wrong — **face the direction of movement**; move with feet, not the waist. **Pull** is a distractor: **push** with the whole body, arms close.
- **Bending at the waist with straight legs** ("lift with your back") is a classic wrong answer — flex the knees and hips, keep the object close so legs/arms bear the load; back muscles are weaker and injury-prone.
- **Narrow base of support** or standing far from the object = unstable (line of gravity outside base). Correct: feet **shoulder-width**, knees/hips flexed, object close to COG.
- **Degree confusion:** Fowler's = **45–60°**; high-Fowler's = **90°**; low/semi-Fowler's = **30°**. Test questions will swap these.
- **Picking semi-Fowler's for a patient at risk for skin breakdown** — avoid it; even a few centimeters of head elevation increases **sacral shearing**. Also remember Fowler's pressure points: heels, sacrum, scapulae (and buttocks bear weight).
- **Prone position** is a trap for patients with **spinal problems** (gravity produces lumbar lordosis) and requires foot positioning/footboard to prevent **plantar flexion/footdrop**. Trochanter rolls prevent **external hip rotation**; footboards prevent footdrop.
- **"Reposition every 2 hours for every patient"** is too rigid — frequency is **individualized**, reevaluated, and documented on a schedule when the patient cannot move independently.
- **Dementia handling distractors:** long sentences, "Don't…" (negative) instructions, quizzing multiple questions at a time, rushing, correcting the process mid-task. Correct: face the patient,

name, slow calm speech, short **positive** one-step commands, allow response time, work **with** established habits; check for **receptive aphasia**.

- **Manual lifting without assistive devices** (or "just get more staff and lift") is unsafe — use friction-reducing sheets, gait belts, standing/lateral assist devices, powered full-body lifts; screen first, get enough staff, lock wheels, elevate bed, give analgesia before movement.
- **Do not let the patient "do nothing"** — encourage the patient to assist within capability to promote independence and self-esteem; likewise, **ADL ≠ IADL** (finances, transportation, meal prep, housekeeping are IADLs).

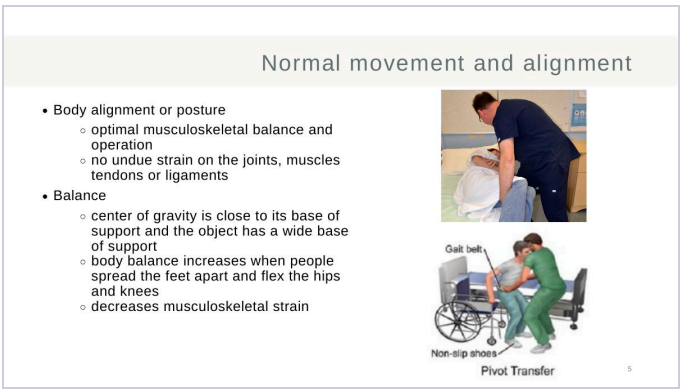
Her slides: pictures



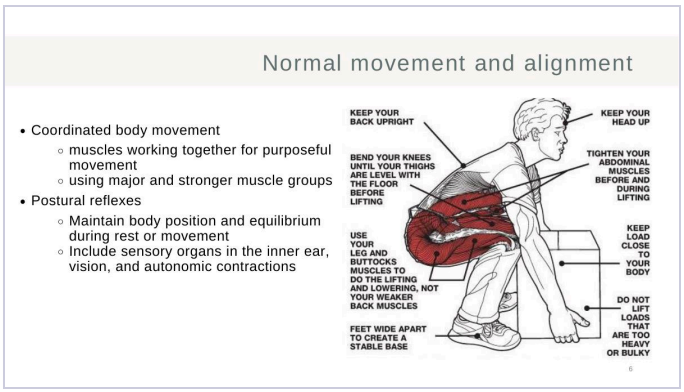
Ch. 34 slide 1



Ch. 34 slide 2



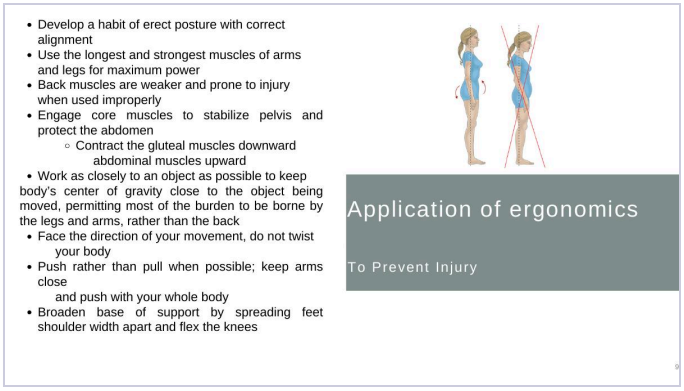
Ch. 34 slide 3



Ch. 34 slide 4



Ch. 34 slide 6



Ch. 34 slide 7

What Leads to patient handling injuries?

- Uncoordinated lifts
- High exertion while in an awkward position

- Manual lifting and transferring without assistive devices
- Lifting while fatigued

- Repetitive movements such as lifting, transferring, and repositioning patients
- Standing for long periods

- Transferring patients from beds to gurneys or wheelchairs
- Repositioning patients in bed

- Transferring or repositioning patients with cognitive impairments, who are dependent or obese or who are unable or unwilling to actively participate and promote their own movements

Ch. 34 slide 10

Devices to aid in mobility



Gait belts

Standing assist



Lateral assist Device

Friction- reducing sheets



Powered full-body lifts

Ch. 34 slide 11

Patient positioning







Ch. 34 slide 12

Modules 2–3 · Ch. 26 Vital Signs

Instructor slides (PRIMARY — study these first)

Objectives & overview

- Learning objectives: explain physiologic processes of homeostatic regulation of temperature, pulse, respirations, and BP; compare/contrast factors that increase or decrease each; identify assessment sites for temperature, pulse, and BP; know normal adult ranges for T, P, R, BP; document VS accurately. [Slide 2]
- Vital signs are indicators of physiologic functioning and reflect a person's health status. Vital signs = **temperature, pulse, respirations, blood pressure, pulse oximetry, pain level**. [Slide 3]

When to assess vital signs [Slide 4]

- On admission to any health care facility or institution.
- Based on facility or institutional policies and procedures.
- Any time there is a change in the patient's condition.
- Any time there is a loss of consciousness.
- Before and after any surgical or invasive diagnostic procedure.
- Before and after activity that may increase risk (e.g., ambulation after surgery).
- Before administering medications that affect cardiovascular and respiratory function.
- Whenever the patient reports a change in status, pain, or distress.

Temperature — physiology & factors

- **Hypothalamus** controls thermoregulation by conserving body heat or increasing heat loss; body temperature = difference between heat produced and heat lost to the environment. [Slide 5]
- Factors affecting body temp: time of day, biologic sex, age, physical activity, state of health, environmental temperatures. [Slide 5]
- **Core body temp locations:** 1) intracranial, 2) intrathoracic, 3) intra-abdominal. Tissues/cells function best between **36–37.5 °C (98.6 °F)**. [Slide 5]
- **Circadian rhythm:** predictable fluctuations — lower in the morning, **peak between 1500–1800**. [Slide 6]
- Older adults tend to run **lower** and have **decreased** sensitivity to temperature change; women have greater fluctuation (hormonal). [Slide 6]
- Physical activity and health status alter temp; extreme cold → hypothermia, extreme heat → hyperthermia. [Slide 6]

Fever (pyrexia) & hypothermia

- Increased temperature = **febrile**; no fever = **afebrile**. Extremely high = **above 40 °C (>104 °F)**, dangerous. **≥ 41 °C (106 °F) = medical emergency = hyperpyrexia**. [Slide 7]
- Fever S/Sx: loss of appetite, headache, hot dry skin, thirst, muscle aches & fatigue. [Slide 7]
- Key relationships: core temp is higher than body surface temp; most accurate core sources are intracranial/intrathoracic/intra-abdominal (rectal is the practical core-reflective route); **pulse rate rises with pyrexia**. [Slide 8]
- Tx of pyrexia: fever is a sign of immune response and an important defense mechanism, but treatment aims at **comfort and preventing complications**. Pharm: antipyretics; treat

bacterial infection. Non-pharm: cool bath, cooling blankets, removing excess clothing/blankets, **oral fluids to prevent dehydration**. [Slide 9]

- Hypothermia S/Sx: poor coordination, slurred speech, amnesia, hallucinations, stupor; respirations, HR, and BP **decrease**. Tx: raise body temp / promote warmth — warm blankets, warm fluids, **Bair Hugger**. [Slide 10]

Thermometers & temperature sites

- Device types: infrared, tympanic, electronic (oral/axillary), rectal electronic, attachments. [Slide 11]

Site	Positioning / technique	Time in place	Relation to oral	Cautions
Oral	Posterior sublingual pocket (under tongue, close to carotid artery); patient awake & alert	3 min	Reference	No hot/cold drinks or smoking 20 min prior; not for small children (bite down)
Axillary	Bulb in center of axilla; lower arm positioned across chest; non-invasive (good for children)	5–10 min	**0.5 °C lower**	Less accurate (no major vessels nearby)
Rectal	Side-lying, upper leg flexed; insert lubricated bulb 1–1½ inch (adult)	2–3 min	**0.5 °C higher**	Used when oral route unsafe/inaccurate (unconscious, disoriented, irrational)
Ear (tympanic)	Close to hypothalamus (sensitive to core temp changes); adult: pull pinna **up & back**	2–3 seconds	—	Rapid, easy access; **cerumen impaction** and **otitis media** distort readings

[Slide 12]

Pulse

- Left ventricle contracts → wave of blood surges through arteries; felt by palpating an artery lightly against underlying bone or muscle. Sites: carotid, brachial, radial, femoral, popliteal, posterior tibial, dorsalis pedis. Assess **rate, rhythm, amplitude** by palpation and auscultation. [Slide 13]
- Procedure: peripheral pulse — use 2nd, 3rd, or 4th fingers lightly over the artery; **do NOT use your thumb**. Count 30 sec × 2; **if irregular, count radial for 1 min and then apically for a full minute**. Apical = beat at the apex/PMI; auscultate a full minute (“lub-dub” = one beat). **Auscultate apical, palpate peripheral.** [Slide 14]
- Rate: **60–100 bpm** (average 80). **Tachycardia > 100 bpm; bradycardia < 60 bpm.** Rhythm: regular or irregular. Amplitude: volume of blood pushed against the arterial wall during ventricular contraction — weak/thready (lacks fullness), full/bounding (higher than normal), imperceptible (cannot be felt or heard). [Slide 15]

Respirations

- Physiology: gas exchange between oxygen and carbon dioxide; controlled by **autonomic and voluntary** mechanisms. Inspiration = breathing in; expiration = breathing out. [Slide 16]
- Assess by observing **rate, rhythm, and depth** — observe **after taking the pulse, while still holding the patient’s hand, so the patient is unaware** you are counting respirations. [Slide 16]
- Documentation terms: increased rate = **tachypnea**; decreased rate = **bradypnea**; periods of no breathing = **apnea**; difficulty/labored breathing = **dyspnea**; difficulty breathing lying down =

orthopnea. [Slide 16]

- **Rate:** one inhale + one exhale = one cycle; normal adult = **12–20 breaths/min**. Abnormal increase = tachypnea; decrease = bradypnea; absence = apnea. [Slide 17]
- **Depth:** amount of air inhaled/exhaled — normal (deep and even chest movements), shallow (minimal rise/fall), SOB (shallow and rapid). [Slide 17]
- **Rhythm:** regularity of inhalation/exhalation; normal shows very little variation in length of pauses between I & E. **Character:** adventitious deviations from normal effortless breathing; dyspnea = difficult or labored breathing. [Slide 17, Slide 18]

Blood pressure

- BP = force exerted by the blood against vessel walls. **Systolic** = highest pressure during contraction; **diastolic** = lowest pressure on artery walls during relaxation. [Slide 19]
- Factors affecting BP: position changes (**orthostatic hypotension is common in the elderly**), drugs/medications, food, exercise, age, weight, biologic sex, race, stress. [Slide 19]

Category	Values (per slide)
Normal	< 120/80
Hypotension	readings lower than 90 mm Hg systolic or 60 mm Hg diastolic
Prehypertension (elevated)	Systolic 120–129 and diastolic of 80
Hypertension Stage 1	130–139 systolic, 80–89 diastolic
Hypertension Stage 2	> 140 systolic, > 90 diastolic
Hypertensive Crisis	> 180

[Slide 20]

- BP assessment: correct **cuff size judged by arm circumference, NOT age**. Cuff should cover **40% of the arm circumference**; the bladder should cover **2/3 of the arm**. Support arm at **heart level, palm turned upward**; arm **above** heart → **false LOW** reading. [Slide 21]
- Cuff errors: **too wide → false low; too narrow → false high; too loose → false high.** [Slide 21]
- **Korotkoff sounds:** series of sounds created as blood flows through an artery after it has been occluded with a cuff and cuff pressure is gradually released. [Slide 21]
- **Limb alerts — do NOT measure on:** PICC lines, arteriovenous (AV) fistula, arm on the side of a mastectomy. [Slide 21]
- (Slide 22 is an illustration of BP technique/cuff application — no additional text points.) [Slide 22]

Pulse oximetry & pain

- **Non-invasive** measurement of oxygen saturation; calculates **SpO2** (pulse oxygen saturation) — a reliable **estimate** of arterial oxygen saturation. Probes: finger, ear, nose, toe. [Slide 23]
- Considerations: remove thick nail polish; **wait until the readout reaches a constant value and the pulse display reaches full strength**; during continuous monitoring, inspect skin under the probe for skin integrity and **rotate the probe**. [Slide 23]
- PQRST pain scale is used for verbal, aware patients (pain level is part of the VS set). [Slide 24]

Application & documentation principles

- Apply findings to a client scenario: identify which VS are outside normal limits; prioritize the top 3 client problems; determine the source of the change in status; identify non-pharmacologic comfort measures (e.g., cooling measures) to use **before** medication administration; when respiratory rate is increased, further assess oxygenation (pulse oximetry). [Slide 25]

- VS may reveal **sudden** changes in a client's condition as well as **progressive** changes over time; a **baseline set of VS** is important to identify changes in the patient's condition. [Slide 26]
- Besides the numbers, consider the patient's symptoms, context, and usual patterns; **a client's normal range can differ from standard values**. [Slide 26]

From the assigned reading (Taylor — details the slides do not cover)

General principles

- VS assessment may be delegated to assistive personnel when the patient is stable, but the **nurse** is responsible for accuracy of data, interpretation, and reporting abnormal findings; validate unexpected findings and assess further. [Taylor p. 717]
- Frequency: hospitalized patients with elevated temps, high/low BP, pulse rate/rhythm changes, or respiratory difficulty are assessed **at 4-hour intervals at minimum**; long-term care requires monthly VS (daily for Medicare A skilled residents); critical care uses continuous monitoring. [Taylor p. 717]
- VS plus oxygen saturation are identified as the best indicators of cardiopulmonary arrest, unplanned ICU admission, and unexpected death; trends over time aid early detection of deterioration. [Taylor p. 716]
- Normal values vary across the life span (Table 26-1); always compare to the patient's own baseline. [Taylor Table 26-1]

Temperature

- Normal body temperature ranges **96.7–100.5 °F (35.9–38 °C)** depending on route. [Taylor p. 718]
- Heat production: metabolism (primary), epinephrine/norepinephrine (rapid), thyroid hormone (slow), shivering, piloerection ("goose bumps" reduces surface area for heat loss), physical exertion. [Taylor p. 719]
- Heat loss: skin is the primary site; **arteriovenous shunts** open to dissipate or close to retain heat (sympathetic control). Mechanisms of heat transfer: **radiation, convection, evaporation, conduction**. [Taylor p. 719; Taylor Table 26-2]
- Circadian variation can be up to 1.0 °F (0.5 °C)/day; peak elevation occurs **3–6 PM**; progesterone at ovulation raises a woman's temp 0.5–1.0 °F. [Taylor p. 719]
- Fever results from an upward displacement of the hypothalamic set point caused by **pyrogens** (bacteria, bacterial products, viruses, chemicals from tissue injury — MI, PE, cancer, trauma, surgery); rising mechanisms are shivering, piloerection, vasoconstriction, increased metabolism. [Taylor p. 720]
- Fever patterns (Box 26-2): **intermittent** (returns to normal ≥ once/24 h), **remittent** (never normal, fluctuates), **sustained/continuous** (above normal with minimal variation), **relapsing/recurrent** (normal for ≥1 day between episodes). [Taylor Box 26-2]
- **Hyperthermia differs from fever:** hypothalamic set point is NOT changed — cooling mechanisms are simply overwhelmed. **Neurogenic fever** (hypothalamic damage) does not respond to antipyretics. **FUO** = ≥ 38.3 °C (101 °F) for ≥ 3 weeks without identified cause. [Taylor p. 720]
- Fever complications: febrile seizures (infants/children), confusion/delirium (older adults), fever blisters, fluid/electrolyte/acid–base imbalances. Older adults run a lower baseline — fever may be a **late** sign and only slightly above normal even in serious infection; mild elevation in infants < 3 months may signal serious infection. [Taylor p. 720–721]
- Antipyretics (aspirin, ibuprofen, acetaminophen) reset the set point; **routine antipyretic administration is usually avoided**; **no aspirin or aspirin-containing products for**

anyone < 19 years with fever/flu-like illness (Reye syndrome). Non-pharm: cool sponge baths, cool packs, cooling (hypothermia) blankets, oral fluids, simple carbohydrates. [Taylor p. 721]

- **Hypothermia = core temp below 35 °C (95 °F)**; risks: unprotected cold exposure, older adults/disabled, alcoholism, malnutrition, hypothyroidism, perioperative patients, neonates. **Therapeutic hypothermia** (purposeful cooling) improves neurologic outcomes after brain injury, cardiac arrest, some surgeries. [Taylor p. 721]
- Hypothermia blanket (Guidelines 26-1): protect skin with lanolin/cold cream, cover pad with a sheet, insert rectal/esophageal/axillary probe, elevate scrotum off the pad, turn q30 min–1 h, monitor VS/neuro checks (often q15 min), watch for shivering, discontinue ~1 degree above desired temp. [Taylor p. 722–723]
- Equipment: mercury glass thermometers are not used in facilities; shared/nondisposable thermometers require cleaning/disinfection (cross-contamination risk); noncontact infrared thermometers are for **fever screening, not ongoing monitoring**. [Taylor p. 723–724]
- **Always document the site**; if no site is documented, it is assumed to be **oral**. Comparing two different sites is a way to double-check an unusual reading. [Taylor p. 725]
- Oral: contraindicated in children < 6 years, unable to follow directions, oral mucosa inflammation/surgery, confused/comatose, mouth breathing, seizure risk, and **patients on oxygen by mask**; wait 15–30 min after hot/cold food/fluids or smoking. Normal oral 96.6–99.5 °F. [Taylor p. 725]
- Tympanic: do not use with ear drainage, ear pain, ear infection, or tympanic scarring — use the **opposite ear**; cerumen affects readings. Normal tympanic 98.2–100.9 °F. [Taylor p. 725]
- Temporal artery: head coverings/hair/wigs insulate → **falsely high**; measure only the exposed side if lying on one side; do not measure over scar tissue, lesions, abrasions; diaphoresis impairs accuracy. Normal 98.7–100.5 °F. [Taylor p. 725–726]
- Axillary: less accurate — do not use when accuracy is required; most common site in the **neonate**; hold arm to the side until complete. Normal 95.6–98.5 °F. [Taylor p. 726]
- Rectal: one of the most accurate/core-reflective routes, BUT contraindicated in **newborns, children with diarrhea, recent rectal/anal/vaginal/prostate surgery, rectal disease, neutropenia, thrombocytopenia, certain neurologic disorders (spinal cord injury)**; vagal stimulation may slow the heart rate (avoid in some cardiac patients). Normal 97.4–100.5 °F. [Taylor p. 726]

Pulse

- Pulse is regulated by the autonomic nervous system through the **SA node**; vagal (parasympathetic) stimulation slows HR; sympathetic stimulation increases HR and force of contraction. [Taylor p. 727]
- Factors affecting pulse (Table 26-4): age (older adults may have decreased rate), biologic sex (women slightly higher), physical activity (athletes may be below normal), body temperature (fever ↑, hypothermia ↓), stress/pain/fear/anxiety, medications (↑ or ↓), disease/health conditions (COPD, pneumonia, sepsis, PVD, heart failure). [Taylor Table 26-4]
- Tachycardia causes (Box 26-3): ↓ BP (blood loss, shock, trauma), elevated temperature, poor oxygenation (chronic pulmonary disease, anemia), exercise, hyperthyroidism, pain, strong emotions, some drugs (atropine, epinephrine). Bradycardia causes: CAD/MI, decreased temperature, hypoxemia/OSA, delirium, hypothyroidism, increased ICP/CVA, vagal stimulation (ocular pressure, vomiting), drugs (digitalis, antiarrhythmics). [Taylor Box 26-3]
- Pulse amplitude grading (Table 26-5): **0** absent; **+1** diminished; **+2** normal/brisk; **+3** bounding. Bounding with early septic shock, exercise, fever, fluid volume excess, anxiety; weak/thready with heart failure, hypovolemia, shock, hypernatremia, arrhythmias. [Taylor Table 26-5, p. 728]

- **Pulse elasticity** (a 4th characteristic): normal artery feels smooth, straight, springy; loses elasticity with age. [Taylor p. 728]
- Radial pulse is hard to palpate accurately in children < 2 years — **auscultate apical pulse for 1 full minute**. Carotid: palpate **one side at a time** (cerebrovascular safety), used in shock/cardiac arrest; **brachial** is used for infants in emergencies. [Taylor p. 730]
- Apical pulse site: **5th intercostal space, left midclavicular line**; alternative Erb's point (2nd–3rd ICS left sternal border); count a full minute. Required when peripheral pulse is irregular, weak, or very rapid, and **before giving medications that alter heart rate/rhythm**. [Taylor p. 730–731]
- **Apical–radial pulse (2 nurses, 1 shared watch, both count 1 full minute)**: the difference is the **pulse deficit** — indicates not all beats reach the periphery/insufficient cardiac force → decreased tissue perfusion. [Taylor p. 731–732]
- Stethoscope: **diaphragm** = high-frequency sounds (lungs); **bell** = low-frequency sounds (heart/vessels). Doppler ultrasound used for pulses too weak to palpate (and for systolic BP estimate). [Taylor p. 729, 742]

Respirations

- Respiration = **ventilation** (movement of gases in/out), **diffusion** (gas exchange alveoli ↔ blood), **perfusion** (blood ↔ tissue cells). Only ventilation is measured as a VS. Normal unlabored breathing = **eupnea**; ~1 respiration per 4 heartbeats in healthy people. [Taylor p. 731, 733]
- Increased CO_2 is the most powerful respiratory stimulant; cerebral cortex gives voluntary control. [Taylor p. 732]
- Factors (Box 26-4): age (newborn 30–60 → adult 12–20), exercise, acid–base imbalance (acidosis → hyperventilation), brain lesions (Cheyne–Stokes), altitude, respiratory disease/smoking, anemia, anxiety (sighing respirations), medications (narcotics/sedatives/anesthetics slow rate & depth; amphetamines/cocaine increase), acute pain (↑ rate, ↓ depth). [Taylor Box 26-4]
- Abnormal patterns (Table 26-6): **tachypnea > 24 breaths/min, shallow**; **bradypnea < 10 breaths/min, regular**; **hyperventilation** = increased rate and depth (fear, exercise, DKA/Kussmaul); **hypoventilation** = decreased rate and depth, irregular (narcotic/anesthetic overdose); **Cheyne–Stokes** = alternating deep rapid breathing with apnea (drug overdose, HF, increased ICP, renal failure); **Biot's** = varying depth/rate with periods of apnea, irregular (meningitis, severe brain damage). [Taylor Table 26-6]
- **Apnea** without intervention → death within a few minutes. **Orthopnea** = breathes more easily upright; SOB when lying flat (pulmonary edema, heart disease, COPD). **Dyspnea** is perceived only by the person experiencing it. [Taylor p. 734]

Blood pressure

- **Pulse pressure = systolic − diastolic** (e.g., 120/80 → pulse pressure 40 mm Hg). Cardiac output and peripheral resistance determine systolic and diastolic pressures. [Taylor p. 734]
- Short-term regulation: neural (cardiovascular center, vagus, sympathetic) + humoral (epinephrine, renin–angiotensin–aldosterone → vasoconstriction, Na/water retention; ADH/vasopressin). Long-term: renal regulation of extracellular fluid volume. **Baroreceptors** (aortic arch, carotid) sense vessel stretch. [Taylor p. 734–735]
- Factors in healthy people (Box 26-5): age (↓ arterial elasticity), circadian rhythm (lowest on arising; rises 5–10 mm Hg by late afternoon), biologic sex (women lower until menopause), food intake (↑ after eating), exercise (↑ systolic), weight (↑ with obesity), emotional state (anger, fear, excitement, pain ↑), body position (lower supine/prone than sitting/standing), race (hypertension more prevalent/severe in African American adults), drugs (oral contraceptives ↑). [Taylor Box 26-5]

- A rise or fall of **20–30 mm Hg from a person's own baseline is significant** even if within the normal range. A single elevated reading is not necessarily significant; hypertension is diagnosed on the **average of 2+ readings on 2+ occasions**. [Taylor p. 735]

Category (adults ≥ 18)	Systolic		Diastolic
Normal	< 120	**and**	< 80
Elevated	120–129	**and**	< 80
Hypertension Stage 1	130–139	**or**	80–89
Hypertension Stage 2	≥ 140	**or**	≥ 90
Hypertensive crisis	> 180	**and/or**	> 120

[Taylor Table 26-7]

- Hypertension is a major risk factor for CVA, MI, heart failure, PVD, abdominal aortic aneurysm ("silent killer"). Treatment: lifestyle changes (weight loss, reduced sodium/saturated fat, ↑ fruits/vegetables/grains, limit alcohol, stress management, regular activity) plus antihypertensives (diuretics, ACE inhibitors, ARBs, vasodilators/CCBs, beta-blockers). [Taylor p. 736–737]
- **Hypotension = BP lower than 90/60 mm Hg.** Concern when symptomatic: dizziness, tachycardia, pallor, syncope, diaphoresis, blurred vision, nausea, confusion — report immediately. [Taylor p. 737]
- **Orthostatic (postural) hypotension:** sustained drop of **≥ 20 mm Hg systolic or ≥ 10 mm Hg diastolic within 3 minutes of standing**; also delayed type (> 3 min) and initial type (transient ≥ 40 systolic/≥ 20 diastolic within 15 sec). Causes: dehydration/blood loss, neurologic/cardiovascular/endocrine problems, medications. **Older adults at increased fall risk.** [Taylor p. 737–738]
- Orthostatic assessment (Guidelines 26-5): supine 3–10 min → baseline BP/pulse; sit on edge with legs dangling 1–3 min → BP/pulse; stand, measure at 30 sec, 60 sec, and up to 3 min. Return supine immediately if dizziness/pallor/diaphoresis/syncope; do not proceed to standing if symptomatic when sitting. Dangle before standing after bed rest. [Taylor p. 738]

Cuff selection & measurement technique

- Cuff bladder **length 75%–100%** of arm circumference, **width 37%–50%** (ratio 2:1); sizes: small adult 12 × 22 cm (arm 22–26 cm), adult 16 × 30 (27–34 cm), large adult 16 × 36 (35–44 cm), thigh 16 × 42 (45–52 cm). **Incorrect cuff size is the most frequent BP error**. Cuff too narrow → falsely high; too wide → falsely low. Cuff taut: one finger fits easily at top/bottom, two fingers very snug. [Taylor Table 26-8, p. 739–740]
- Patient prep: quiet environment; **no caffeine, exercise, or smoking for 30 min before**; no talking or moving; back supported (no back support ↑ systolic up to 15 mm Hg, diastolic up to 6); **legs uncrossed** (crossed ↑ systolic up to 8 mm Hg); arm supported at heart level (midpoint of sternum). Supine readings may run up to 10 mm Hg systolic/5 mm Hg diastolic higher than seated. [Taylor p. 740]
- Errors (Box 26-6): noncalibrated/nonvalidated device; wrong cuff size; patient movement/talking; incorrect cuff placement/orientation; incorrect patient or limb positioning; poor technique; **inflating too rapidly**; **deflating faster than 2–3 mm Hg/second**; environmental noise; no rest period; incorrect interpretation; relying on a single-occasion reading. [Taylor Box 26-6]
- Methods: auscultatory (sphygmomanometer + stethoscope), **oscillometric** (automated, now common; devices require calibration/validated use), Doppler (estimate of **systolic only**, when

sounds inaudible), direct arterial line (ICU), ambulatory (ABPM) and home (HBPM) monitoring. [Taylor p. 741–743]
- BP sites: brachial and popliteal most common; forearm/radial for very large or obese arms; wrist or ankle when no cuff fits. [Taylor p. 743]

Korotkoff sounds (auscultatory method)

Phase	Description
I	First appearance of faint but clear **tapping** sounds that gradually increase in intensity — **first tapping sound = systolic pressure**
II	**Muffled or swishing** sounds; may temporarily disappear — the **auscultatory gap** (up to 40 mm Hg). Missing it → underestimating systolic or overestimating diastolic
III	Distinct, **loud** sounds as blood flows relatively freely through an increasingly open artery
IV	Distinct, abrupt **muffling** with a soft, blowing quality
V	**Last sound heard before loss of all sounds = diastolic pressure**

Read to the nearest even number (nearest 2 mm Hg). If sounds are heard all the way to zero (pregnancy, AV fistula), record Phase IV as well (e.g., 112/62/0). [Taylor Table 26-9, p. 742]

Pain & pulse oximetry (context)

- Pain assessment is routinely included with VS (discussed in depth in Ch. 36); use PQRST for verbal, aware patients. [Taylor p. 716; Slide 24]
- Pulse oximetry is the noninvasive measurement of arterial oxyhemoglobin saturation (SpO2) — detailed in Ch. 40; it complements, but does not replace, full respiratory assessment and ABG monitoring. [Taylor p. 716, 734]

⚠ **Conflict:** Elevated/prehypertension category — Slide 20 lists “systolic 120–129 and diastolic of 80,” while Taylor Table 26-7 defines **Elevated = systolic 120–129 AND diastolic < 80**. **Go with the slide wording for her exam**, but know the AHA/Taylor table for NCLEX. [Slide 20; Taylor Table 26-7]

⚠ **Conflict:** Hypertensive crisis threshold — Slide 20 states “> 180” (systolic only), while Taylor Table 26-7 states **> 180 systolic and/or > 120 diastolic**. Go with the slides for her exam. [Slide 20; Taylor Table 26-7]

⚠ **Conflict:** Cuff bladder coverage — Slide 21 states the bladder should cover “2/3 of the arm,” while Taylor Table 26-8 recommends bladder length of **75%–100% of arm circumference** (width 37%–50%, ratio 2:1). Go with the slides for her exam. [Slide 21; Taylor Table 26-8]

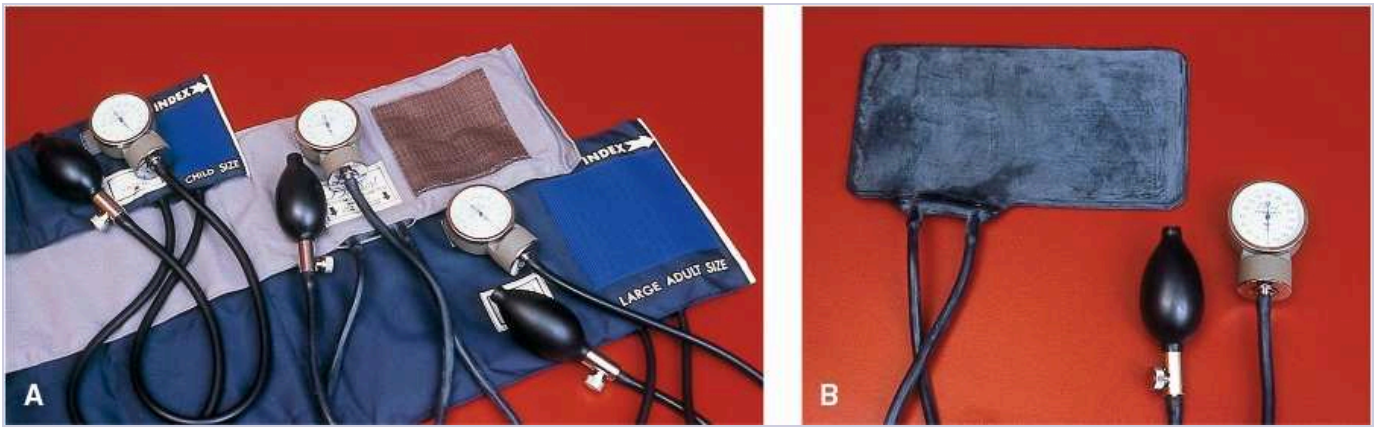
NCLEX test traps

- **Thumb vs. fingers:** never palpate a pulse with your thumb (it has its own pulse) — use the pads of the 2nd, 3rd, or 4th fingers. Also: 30 sec × 2 is acceptable **only for a regular** rhythm; an irregular rhythm requires a full 60-second count (radial, plus apical for a pulse deficit).
- **BP category logic (and vs. or):** Normal and Elevated require the systolic **AND** diastolic criteria to be met; Hypertension Stage 1/2 are met by the systolic **OR** diastolic value (e.g., 118/92 = Stage 1, not “normal”).
- **Cuff size errors:** cuff **too narrow/too small → falsely HIGH**; cuff **too wide/too large → falsely LOW**. Size by **arm circumference, not the patient’s age**. Too loose = falsely high.
- **Arm position errors:** arm **above** heart level → falsely low; arm **below** heart level → falsely high. Unsupported back, crossed legs, talking, recent caffeine/smoking/exercise, and recent activity all raise readings. No back support can raise systolic up to 15 mm Hg.

- **Korotkoff phases:** Phase I = systolic; **Phase V (not IV) = diastolic**. The **auscultatory gap** (Phase II) can span up to 40 mm Hg — missing it underestimates systolic or overestimates diastolic. Deflate at 2–3 mm Hg/second and read to the nearest 2 mm Hg. In pregnancy or with AV fistulas, sounds may persist to zero — document Phase IV too.
- **Limb selection:** avoid the arm with a PICC line, an AV fistula, or a mastectomy; use the other arm, forearm/wrist, or ankle instead.
- **Respirations counted "out loud":** tell the patient you are counting the pulse, keep the hand on the wrist, and observe chest rise/fall covertly — a patient who knows respirations are being counted will change the rate.
- **Temperature site math and contraindications:** axillary = oral − 0.5 °C; rectal = oral + 0.5 °C. Oral is contraindicated in children < 6 years (bite risk), unconscious/confused patients, mouth breathers, after hot/cold intake or smoking, and in patients on an oxygen mask. Rectal is contraindicated in newborns, diarrhea, thrombocytopenia/neutropenia, recent rectal/prostate surgery, and some cardiac/neurologic patients. Tympanic is distorted by cerumen, otitis media, and ear drainage (use the opposite ear).
- **Fever numbers:** > 40 °C (

AGE	TEMPERATURE °F °C	PULSE beats/min	RESPIRATIONS breaths/min	BLOOD PRESSURE mm Hg
Newborns	97.2–99.9 36.2–37.7	95–170	30–60	60–70/40
Infants	96–99.7 35.6–37.6	85–170	30–50	85/37
Toddler	96–99 35.6–37.2	70–150	20–40	88/42
Child	96–99 35.6–37.2	65–130	15–25	95/57
Adolescent	96.4–99.5 35.8–37.5	60–115	12–20	102/60
Adult	96.4–99.5 35.8–37.5	60–100	12–20	<120/80

Taylor Table 26-1: Age-related variations in normal vital signs (p. 718)

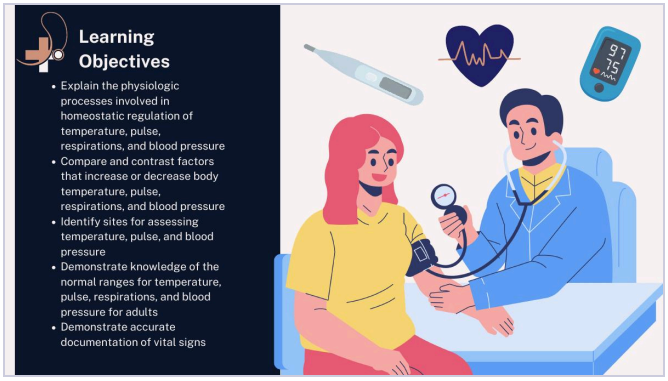


Taylor Table 26-9: Korotkoff sounds

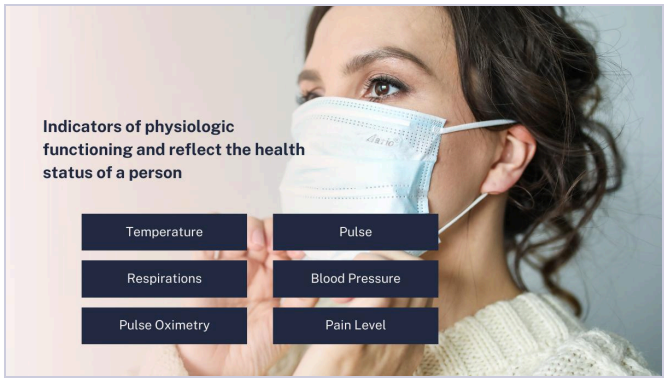
Her slides: pictures



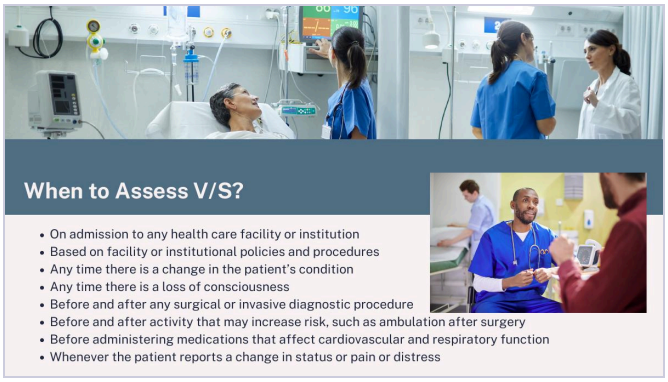
Ch. 26 slide 1



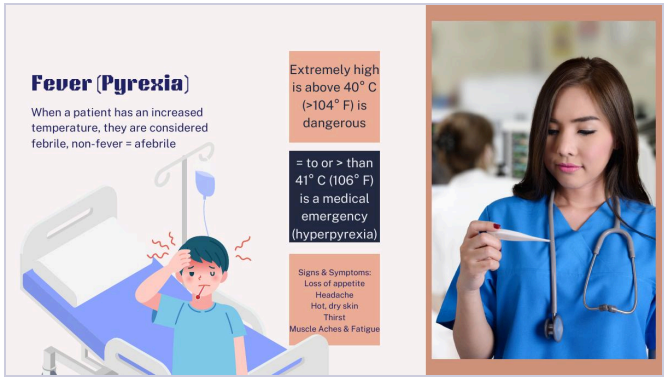
Ch. 26 slide 2



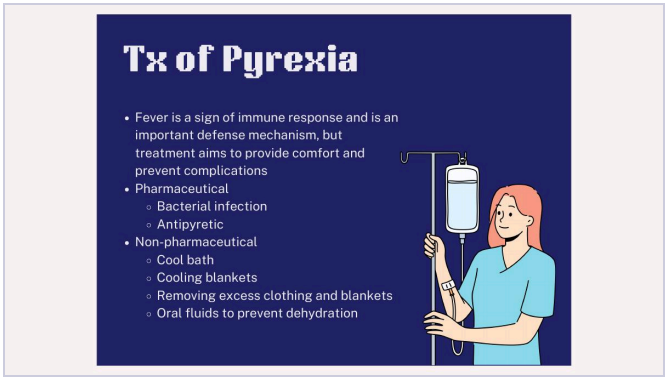
Ch. 26 slide 3



Ch. 26 slide 4



Ch. 26 slide 7



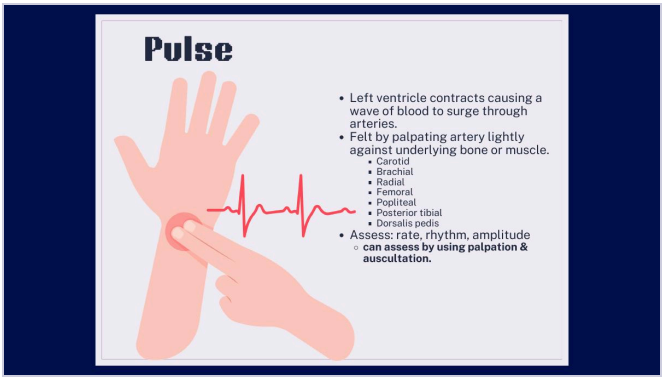
Ch. 26 slide 9



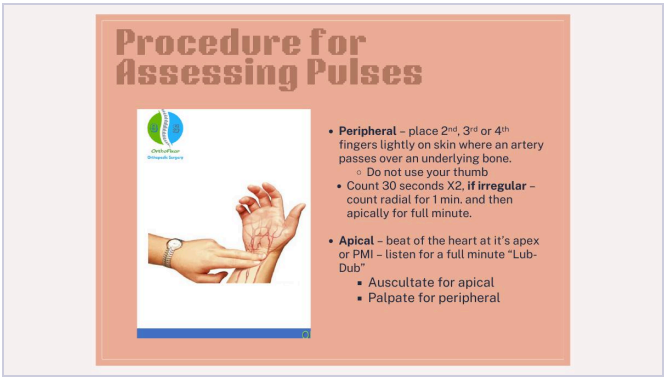
Ch. 26 slide 10



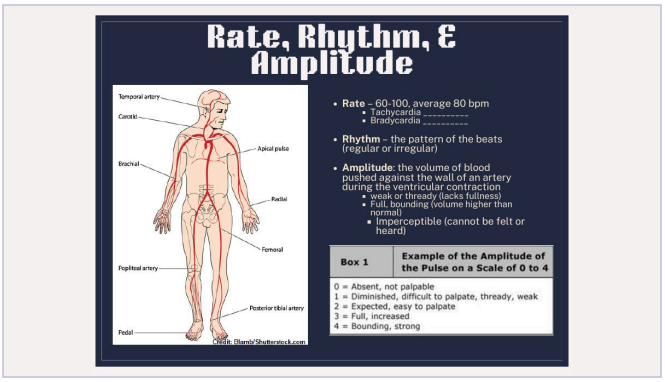
Ch. 26 slide 11



Ch. 26 slide 13



Ch. 26 slide 14



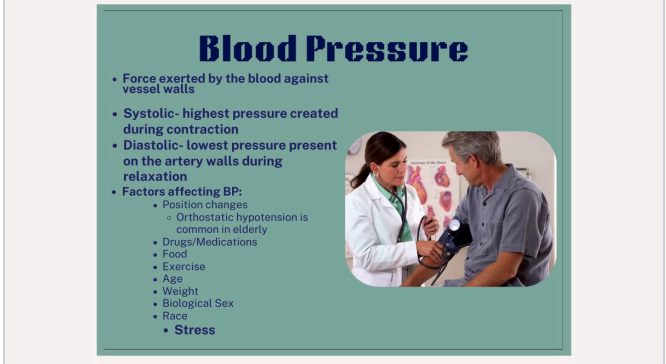
Ch. 26 slide 15



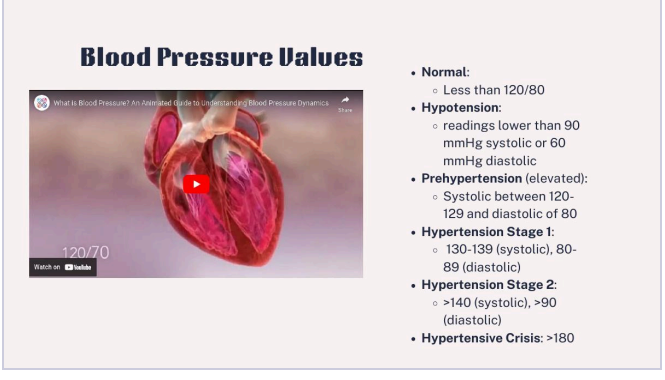
Ch. 26 slide 16



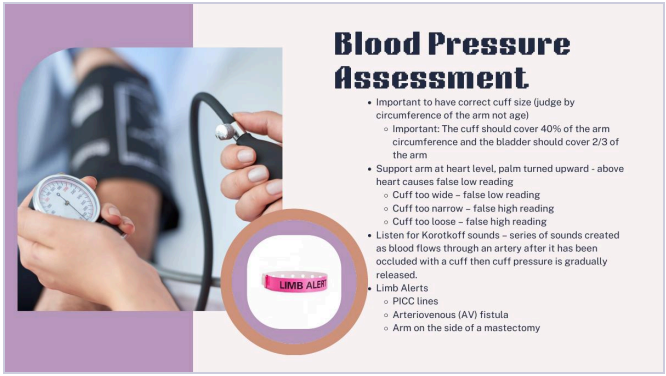
Ch. 26 slide 18



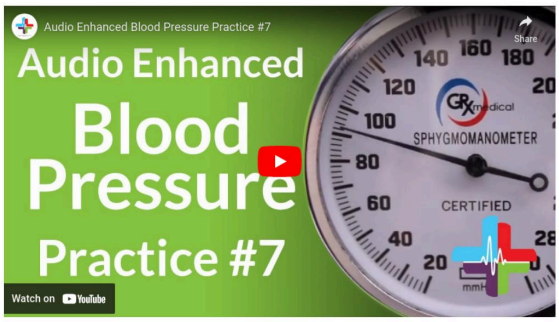
Ch. 26 slide 19



Ch. 26 slide 20



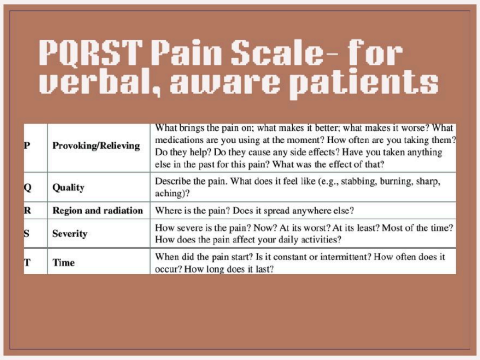
Ch. 26 slide 21



Ch. 26 slide 22



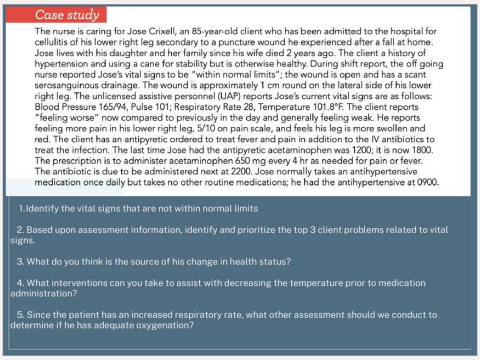
Ch. 26 slide 23



PQRST Pain Scale- for verbal, aware patients

P	**Provoking/Relieving**	What brings the pain on; what makes it better; what makes it worse? What medications are you using at the moment? How often are you taking them? Do they help? Do they cause any side effects? Have you taken anything else in the past for this pain? What was the effect of that?
Q	**Quality**	Describe the pain. What does it feel like (e.g., stabbing, burning, sharp, aching)?
R	**Region and radiation**	Where is the pain? Does it spread anywhere else?
S	**Severity**	How severe is the pain? Now? At its worst? At its least? Most of the time? How does the pain affect your daily activities?
T	**Time**	When did the pain start? Is it constant or intermittent? How often does it occur? How long does it last?

Ch. 26 slide 24



Ch. 26 slide 25

Module 4 · Ch. 8 Communication

Instructor slides: communication foundations

- **Chapter focus:** Communication; Carigan Grino, MSN-Ed, RN. [Slide 1]
- **Learning outcomes:** Describe the communication process, influencing factors, and nonverbal communication; evaluate personal interpersonal competencies; explain ineffective communication; identify skills and barriers with patients and professionals; describe helping-relationship phases; establish therapeutic relationships; adapt care for impaired communication. [Slide 2]
- **Communication:** Exchanging information and generating/transmitting meanings between two or more people. Sending a message does not guarantee successful understanding. Relationship-building, a healthy work environment, safety, and quality care depend on communication. [Slide 3]
- **Process: Sender/encoder** initiates information → **message** is the communication product → **channel** carries it through auditory, visual, or kinesthetic means → **receiver/decoder** receives/interprets it → **feedback** is the receiver's response to the sender. [Slide 5]
- **Verbal:** Spoken communication reliant on language; address language-access needs. **Nonverbal:** Body language, touch, eye contact, facial expressions, posture, gait, gestures, appearance, dress/grooming, sounds, and silence. Notice mismatches between words and behavior; investigate rather than assuming meaning. [Slide 7]
- **Other forms:** Electronic—social media, e-mail, text. Paper—mailed correspondence ("snail mail"). [Slide 11]
- **Nonverbal assessment:** Gait, facial expression, and sounds provide assessment cues; interpret them in context. Practice item not reproduced. [Slide 12]
- **Levels: Intrapersonal** = self-talk, potentially damaging when negative; **interpersonal** = two or more people; **group** = small-group and organizational communication. **Group dynamics:** Identity, cohesiveness, interaction patterns, decision-making, responsibility, leadership, and power. [Slide 13]
- **Influencing factors:** Developmental level; gender; sociocultural differences; space/territory; roles/relationships; occupations/hobbies and communication preferences; physical, mental, and emotional state; values; environment; **trust**. Private zones include head, mouth, feet, face, neck, front of body, and genitalia. [Slide 17]
- **Social relationships:** Spontaneous, mutual exchange; both people's needs considered; equally shared information; no time limits. [Slide 21]
- **Helping/professional relationships:** Assistance with human needs; specific purpose and goals; three phases; definite conclusion; unequal information sharing. Keep disclosure relevant to the patient's needs rather than meeting the nurse's personal needs. [Slide 22]
- **Dispositional traits:** Warmth/friendliness, openness, respect, empathy, honesty, authenticity, trustworthiness, caring, and competence. **Empathy** views the situation from the patient's perspective. **Sympathy** shifts attention toward the nurse's feelings/personal concerns and projects them onto the patient. [Slide 24]
- **Therapeutic conversation:** Appropriate tone; flexibility; clear wording; avoid confusing words; be truthful and open-minded; notice openings for discussion. **Active listening:** Sit, stay alert and relaxed, converse naturally, and use eye contact when appropriate. [Slide 25]

- **Therapeutic presence:** Silence permits free expression. Touch may include a hand on the shoulder or hand; the slide explicitly requires consent for more extensive touch. Humor requires assessment and appropriate use. [Slide 25]
- **Therapeutic questions/responses:** Open-ended, validating, clarifying, and reflective questions; focus on feelings; state observed behaviors; reflect/restate/rephrase; give neutral responses; explore concerns. The slide's reflective examples include reviewing what went well and identifying discharge goals. [Slide 25]
- **Silence:** A communication technique requiring attentive presence, not automatic interruption. Practice item not reproduced. [Slide 32]
- **Avoid:** Closed-ended questions used to limit discussion; forgetting the patient's humanity; failure to listen; leading questions; giving advice; changing the subject; talking about yourself; telling the patient they are wrong; false reassurance; clichés; giving approval; asking "why"; defending other health care team members. These restrict expression, introduce judgment, or redirect attention away from the patient. [Slide 33]
- **Reassurance:** Distinguish supportive communication from unsupported guarantees. Practice item not reproduced. [Slide 40]
- **Communication impairments:** These slides contain headings only in the supplied source; use the reading's specific adaptations below. [Slide 41] [Slide 42] [Slide 43]
- **SBAR: Situation, Background, Assessment, Recommendation.** Standardized nursing communication tool for handoffs and nurse–provider discussions; improves communication effectiveness. [Slide 44]
- **Source limits:** No readable teaching content was supplied for Slides 4, 6, 8–10, 14–16, 18–20, 23, 26–31, 34–39, or 45–46; no image content has been inferred.

Instructor activity slides: additional communication principles

- **Citation key:** The following [Slide N] citations refer to the separate **Communication Scenario in-class activity** deck. Principles are summarized without reproducing scenario questions, completing worksheets, or supplying role-play answer scripts.
- **Purpose:** Twenty therapeutic/nontherapeutic role-plays emphasizing patient-centered communication and clinical judgment. [Slide 1]
- **Method:** Patient, therapeutic nurse, nontherapeutic nurse, and narrator roles; original, realistic, respectful dialogue supported by course materials; explain why approaches differ. [Slide 2]
- **Organization:** Suggested small groups, optional observer/feedback coach, role rotation, 8–10 minutes preparation, 2–3-minute skits, and 1-minute debriefs; stations/sessions may help. **Source arithmetic error:** The stated "20 groups of 4" plus "1 group of 5" equals 85, not the stated 82. [Slide 3]
- **Planning/rehearsal:** Identify roles and concerns; incorporate emotional and nonverbal cues; demonstrate contrasting approaches to the same concern; evaluate patient responses, follow-up communication, realism, respect, and technique clarity. Worksheets are not completed here. [Slide 4] [Slide 5]
- **Reflection versus aggression:** Acknowledge feelings and invite elaboration; defensive/confrontational responses escalate tension. New or worsening symptoms require assessment and escalation, not dismissal as complaints. [Slide 6]
- **Humor versus arguing:** Humor should be gentle, patient-led, and paired with truthful information. Stop if unwelcome; do not debate a patient's fear. [Slide 7]

- **Exploration versus failure to listen:** Attend to meaning and possible safety concerns. Hopelessness may require direct self-harm/suicide assessment, protocol-based action, and team notification. [Slide 8]
- **Open-ended versus accusatory “why”:** Assess understanding without blame; explain in plain language within nursing scope; obtain medical clarification when needed. [Slide 9]
- **Realistic reassurance versus promises:** Acknowledge fear, describe available support, and seek answers; never guarantee outcomes. Refer procedure-specific concerns appropriately. [Slide 10]
- **Listening versus judgment:** Explore treatment barriers such as cost, side effects, routines, and food access before discussing options; avoid labels. [Slide 11]
- **Emotional/spiritual support versus clichés:** Ask about preferred support and relevant practices; do not assume beliefs or try to erase grief immediately. [Slide 12]
- **Reality/reorientation versus minimizing:** Calmly identify yourself, location, purpose, and a simple next step. New confusion requires assessment, prompt reporting, and fall protection —not attribution to normal aging. [Slide 13]
- **Restating/mirroring versus boredom:** Reflect meaning/feelings and allow a response; protect privacy. Sighing, phone checking, clock watching, and dismissiveness communicate disinterest. [Slide 14]
- **Clarifying versus disapproval:** Clarify meaning and priorities; support informed choices without pressure, shame, or scolding; communicate concerns to the appropriate team. [Slide 15]
- **Validation versus advice:** Recognize difficulty without judging ability or taking over. Reports of unsafe feelings, inability to provide care, or thoughts of harm require assessment and escalation. [Slide 16]
- **Direct, focused versus leading questions:** Gather neutral information about location, severity, timing, and functional effects of symptoms. Assess associated findings, follow the care plan, and report concerning changes. [Slide 17]
- **Empathy versus intrusive probing:** Acknowledge distress before sensitive follow-up; build trust, avoid stigma, and do not substitute appearance-based reassurance for assessment. [Slide 18]
- **Touch with permission versus self-focused stories:** Ask before offering touch; respect consent, culture, trauma history, and cues. Presence/listening are alternatives. [Slide 19]
- **Observation versus ignoring cues:** Describe behaviors without accusation. Respect self-report while assessing verbal–nonverbal inconsistencies; do not diagnose pain solely from appearance or stereotypes. [Slide 20]
- **Identity affirmation versus dehumanization:** Ask and consistently use the person’s name/pronouns; acknowledge concerns, protect privacy, and follow documentation policies. Do not make the patient educate the whole team. [Slide 21]
- **Silence versus yes/no-only exchanges:** Brief, attentive silence supports expression; remain present and follow the person’s lead rather than withdrawing. [Slide 22]
- **Collaborative problem-solving versus gossip:** Identify priorities and realistic next steps together. Protect confidentiality and explain necessary safety/support disclosures. Practice exercises do not replace formal support. [Slide 23]
- **Offering self/support versus poor professional presentation:** Provide time, preparation, and practice rather than appearing rushed or distracted. Use teach-back and supervised return demonstration as appropriate; verify adequate instruction and understanding. [Slide 24]

- **Qualified medical interpretation versus frustration:** Speak directly to the patient in short segments; pause for interpretation. Nodding is not proof of understanding. Do not substitute shouting, gestures alone for complex teaching, or a child/unqualified bystander when qualified interpretation is available; follow policy. [Slide 25]
- **Debrief priorities:** Dignity, choices, plain language, listening before advising, verbal/nonverbal awareness without assumptions, and appropriate escalation of clinical/safety concerns. [Slide 26]

From the assigned reading

- **Process refinements:** A patient need stimulates communication. Sender and receiver participate reciprocally and simultaneously. Prior knowledge, experience, feelings, and sociocultural background affect interpretation. **Noise** is anything that distorts a message, including television, pain, or discomfort. Feedback helps verify intended meaning; choose messages requiring minimal decoding effort. [Taylor p. 173] [Taylor p. 175]
- **Verbal/nonverbal distinctions:** Taylor includes **written and spoken words** as verbal communication. Crying, moaning, gasping, and sighing are oral but **nonverbal**. Silence has multiple meanings, including thought, comfort, fear, anger, stoicism, and culturally expected pauses. [Taylor p. 175] [Taylor p. 178] [Taylor p. 193]
- **Cultural interpretation:** Eye contact can convey respect or be experienced as intrusive/confrontational. Gestures vary culturally. Monitor your own expression: disgust or impatience can damage the patient's self-image and trust. Behavior is a cue for assessment, not proof of a specific feeling. [Taylor p. 177]
- **Social media:** Follow HIPAA, professional boundaries, and school/facility policy. Removing names does not guarantee anonymity; room numbers or diagnoses can identify patients. Posts may be redistributed and remain accessible after deletion. Privacy settings do not remove accountability. Report harmful content; separate personal/professional information and help develop online-conduct policies. Consequences may include job loss, board discipline, loss of licensure, and civil/criminal penalties. [Taylor p. 178] [Taylor p. 179] [Box 8-2]
- **E-mail/text:** Use approved safeguards, concise wording, and no text abbreviations. Patient authorization may be required; patient e-mails become part of the health record. **Telehealth:** Professional standards remain unchanged; dress professionally, reduce noise/clutter, ensure privacy, prevent interruptions, and use visible attentive cues. [Taylor p. 179]
- **Positive self-talk and groups:** Self-talk can improve preparation and confidence, not only cause harm. Small groups include meetings, conferences, teaching, and support groups; organizational communication advances institutional goals. Effective groups share goals, trust, two-way communication, problem-solving, responsibility, distributed leadership, and appropriate power use; ineffective groups show alienation, domination, unresolved problems, and power abuse. [Taylor p. 179] [Taylor p. 180] [Table 8-1]
- **Development/roles:** Match language to cognitive development, health literacy, and individual needs. Do not assume a health professional needs no teaching or that an uncomplaining patient has no needs. Territoriality is a person's claim to personal space; home versus institutional settings can change communication. [Taylor p. 180] [Taylor p. 181] [Taylor p. 182]
- **Three relationship phases:**
  - **Orientation:** Introduce names, clarify roles, build trust, orient to the facility, and agree on goals, meeting location/frequency/length, and relationship duration. [Taylor p. 187] [Box 8-

3]
    - **Working/identification and exploration:** Usually longest; active participation, expression of concerns, mutually agreed goals, assistance with daily living, teaching, counseling, and implementation of care. [Taylor p. 187]
    - **Termination/resolution:** Review goals and progress, acknowledge remaining needs, invite feelings about ending, and prepare for the next caregiver/setting. Can occur at shift change, discharge, or nurse departure. Phases may overlap rather than follow a rigid sequence. [Taylor p. 187] [Taylor p. 188]
- **Culturally responsive care:** Examine personal biases; use the patient as an information source; explore individual meanings of health/illness and differences in expectations. Adapt care, pacing, and listening behaviors; validate meaning rather than assuming understanding. Respect reluctance about sensitive topics. Simple repeated wording, pictures, gestures, and appropriate language aids may support communication; interpreters translate meaning, not merely words. [Box 8-4]
- **Rapport builders:** Rapport is mutual trust. Set an interaction objective while staying flexible; provide comfort, privacy, confidentiality, and patient-focused pacing. Explain who will receive information and be honest about available time. Competence includes technical, cognitive, interpersonal, and ethical/legal skills; evaluate your strengths and limitations. [Taylor p. 190] [Taylor p. 191]
- **Additional interviewing skills:** Explain the interview's purpose and plan follow-up. **Sequencing** places events in time and explores possible cause/effect; **directing** returns to an earlier topic or develops a related aspect. Excessive clarification/validation can imply inattentiveness. Listen for repeated themes, avoided topics, gaps, and inconsistencies; think before responding. **Semantics** is the meaning of words. [Taylor p. 192] [Taylor p. 193] [Taylor p. 194] [Taylor p. 195]
- **Assertiveness:** Open, honest, direct, nondefensive communication focused on the **issue, not the person**; use clear "I" statements and respectful posture/tone. Ask for help, admit mistakes, and accept responsibility. Aggression violates others' rights; nonassertiveness/avoidance fails to communicate needs clearly. Taylor references **CUS** as a safety-speaking-up method, but its expansion is not provided in this excerpt. [Taylor p. 195] [Taylor p. 196] [Table 8-2]
- **Additional barriers:** Avoid patronizing names such as "sweetie," stereotypes, intimidating "how" questions, and premature probing. If false reassurance slips out, acknowledge it, apologize, and return to therapeutic communication. [Taylor p. 196] [Taylor p. 197] [Taylor p. 198]
- **Incivility/bullying:** Incivility includes rude/intimidating behavior and failure to assist or share needed information. Bullying is persistent intimidation; covert forms include exclusion, withheld information, rumors, and eye-rolling; overt forms include humiliation and scapegoating. Nurse-to-nurse hostility is also called horizontal/lateral violence. Consequences include distress, burnout, turnover, teamwork failure, and patient-care errors. [Taylor p. 199]
- **Responding to disruption:** Remain calm/professional, address behavior assertively, speak privately when appropriate, and use management/reporting channels. Document facts. Organizations need education, accountability, zero-tolerance policies, reporter protection, leadership role models, and surveillance/reporting systems. Violence is **not an expected part of nursing**. Communication is a first defense, supported by organizational safety measures. Education and conflict-management training can improve difficult conversations. [Taylor p. 199] [Taylor p. 200] [Taylor p. 201]

- **Impaired verbal communication:** Decreased, delayed, or absent ability to use speech. Adapt calmly and individually; use supportive gestures, clarification, and validation. Avoid **elderspeak**—diminutives, exaggerated pitch/intonation, and language implying incompetence. [Taylor p. 201]
- **Visual impairment:** Announce presence/name; speak normally; explain before touching; describe surroundings and sounds; state when leaving; keep call device within reach and glasses/contacts usable. [Box 8-6]
- **Deaf/hard of hearing:** Gain attention, face the patient, use simple sentences and natural speech; keep mouth visible and do not chew gum. Use appropriate demonstration, sign/finger spelling, or writing; check hearing-aid function and placement. [Box 8-6]
- **Physical speech barrier:** Select usable communication methods—blinks/squeezes for yes/no, writing, word/letter/picture boards, flash cards, or sign language. Ensure all caregivers/family understand the method; allow time, reinforce effort, and provide a reliable assistance signal. **Source limit:** Box 8-6 ends at the cognitive-impairment heading; no missing recommendations are inferred. [Box 8-6]
- ⚠ Conflict: **Slide 7** lists verbal communication as spoken; **Taylor p. 175** includes spoken and written words. Use the slide's classification for her exam; recognize Taylor's broader definition.
- ⚠ Conflict: **Slide 33** lists closed-ended questions and advice among techniques to avoid; **Taylor pp. 194, 197–198** permits focused questions for specific data and requested advice within expertise. For her exam, follow the slide's barrier classification; the activity deck also explicitly supports neutral focused assessment questions [activity Slide 17].
- ⚠ Conflict: **Slide 25** labels goal-review prompts "reflective"; **Taylor p. 195** defines reflection as repeating content or describing feelings. Use the instructor's labels on her exam while recognizing both forms in the supplied materials.
- ⚠ Conflict: **Slide 25** explicitly requires consent for touch beyond hand/shoulder contact; **Taylor p. 194** recommends asking before touch generally. The later **activity Slide 19** also says to ask before offering a hand or other touch; follow that explicit slide guidance.

Therapeutic vs non-therapeutic communication

Therapeutic vs non-therapeutic communication

Therapeutic: technique and brief example	Non-therapeutic: technique and brief example to avoid
Clear, truthful, flexible conversation: "I do not know yet; I will find out." [Slide 25]	**Poor professional presentation:** "Make it quick; I have not reviewed anything." [Activity Slide 24]
Active listening: "I'm listening." Sit attentively without interrupting. [Slide 25]	**Failure to listen:** "Uh-huh," while attending to something else. [Slide 33]
Silence: "Take your time," followed by attentive quiet. [Slide 25]	**Nonverbal boredom:** "Go on," while sighing and checking the clock. [Activity Slide 14]
Touch with permission: "May I hold your hand?" [Activity Slide 19]	**Talking about yourself:** "Let me tell you all about my hospital stay." [Slide 33]
Appropriate humor: If the patient jokes about a stubborn sock: "That sock is putting up quite a fight!" [Slide 25; Activity Slide 7]	**Arguing/telling the patient they are wrong:** "You're wrong; there is no reason to feel that way." [Slide 33; Activity Slide 7]
Open-ended question/broad opening: "Tell me what matters most to you today." [Slide 25]	**Closed/yes-no-only exchange:** "Everything okay?" followed by no opportunity to elaborate. [Slide 33; Activity Slide 22]

Therapeutic: technique and brief example	Non-therapeutic: technique and brief example to avoid
Validating: “You said the discomfort started this morning; is that correct?” [Slide 25; Taylor p. 195]	**Leading question:** “You are comfortable now, aren’t you?” [Slide 33]
Clarifying: “What does ‘feeling off’ mean to you?” [Slide 25]	**Intrusive/premature probing:** “Tell me every private detail right now.” [Activity Slide 18; Taylor p. 197]
Reflective question—slide usage: “What would you like to accomplish today?” [Slide 25]	**Accusatory why/how:** “Why didn’t you follow instructions?” “How could you make that choice?” [Slide 33; Taylor p. 197]
Reflecting, restating, rephrasing, mirroring: “The uncertainty feels overwhelming.” [Slide 25; Taylor p. 195]	**Changing the subject:** “Never mind your worries; let’s discuss lunch.” [Slide 33]
Focusing on feelings: “Tell me more about feeling disappointed.” [Slide 25]	**Minimizing concerns:** “That is nothing to be upset about.” [Activity Slide 13]
Stating observations: “I notice your hands are shaking.” [Slide 25]	**Ignoring nonverbal cues:** “You said you’re fine, so there’s nothing else to discuss.” [Activity Slide 20]
Neutral response: “You are considering several options.” [Slide 25]	**Giving approval:** “I approve of that decision.” [Slide 33]
Exploring: “Tell me more about that concern.” [Slide 25]	**Disapproval/judgment:** “That is an irresponsible decision.” [Activity Slides 11, 15; Taylor p. 198]
Empathy: “This sounds difficult for you.” [Slide 24]	**Sympathy that shifts focus:** “Your situation makes me so upset; I cannot stop thinking about my own loss.” [Slide 24]
Realistic reassurance/support: “I will help you get information about the next step.” [Activity Slide 10]	**False reassurance/promises:** “I guarantee nothing will go wrong.” [Slide 33]
Emotional/spiritual support: “What kind of support would be helpful?” [Activity Slide 12]	**Clichés:** “Everything happens for a reason.” [Slide 33]
Presenting reality/reorientation: “You are in the hospital. I’m your nurse.” [Activity Slide 13]	**Dehumanizing/task-only language:** “The gallbladder in room four needs care.” [Slide 33; Taylor p. 196]
Direct/focused question: “When did the dizziness begin?” Use for specific assessment, not to suppress feelings. [Activity Slide 17; Taylor p. 194]	**Defending staff:** “That nurse is excellent; you must have misunderstood.” [Slide 33]
Affirmation/identity validation: “I will use the name and pronouns you use.” [Activity Slide 21]	**Dismissive identity language:** “Your name preference does not matter here.” [Activity Slide 21]
Collaborative problem-solving: “Let’s identify the next step that works for you.” [Activity Slide 23]	**Giving unsolicited advice:** “If I were you, I would choose this.” [Slide 33]
Offering self: “I can sit with you for a few minutes.” [Activity Slide 24]	**Gossip/rumor:** “I heard something about that patient; let me tell you.” [Activity Slide 23; Taylor p. 198]

Therapeutic: technique and brief example	Non-therapeutic: technique and brief example to avoid
Qualified interpreter: “I will arrange an interpreter so we can discuss your care.” [Activity Slide 25]	**Frustration/dismissiveness:** “I already explained it,” said more loudly. [Activity Slide 25]
Teach-back/return demonstration: “Show me how you will use this device.” [Activity Slide 24]	**Assuming understanding:** “You nodded, so you understand.” [Activity Slide 25]
Sequencing: “Did the nausea begin before or after the meal?” [Taylor p. 195]	**Stereotyping/sweeping generalization:** “People your age always complain.” [Taylor p. 197]
Directing: “Let’s return to the sleep difficulty you mentioned.” [Taylor p. 195]	**Patronizing names/elderspeak:** “Sweetie, let’s take our little pills,” in an exaggerated high pitch. [Taylor p. 196] [Taylor p. 201]
Assertive communication: “I am concerned about this change and need you to reassess.” [Taylor p. 195]	**Aggressive behavior:** “Do it my way, or else.” [Activity Slide 6; Taylor p. 196]
Assertive help-seeking: “I need supervision for this procedure.” [Table 8-2]	**Nonassertiveness/avoidance:** “Never mind; I guess I do not need help,” despite uncertainty. [Table 8-2]
Respectful conflict management: “I want to discuss that remark privately.” [Taylor p. 200]	**Incivility/bullying:** “Everyone should see how incompetent you are,” used to humiliate a colleague. [Taylor p. 199]
Communication aids: “Point to the picture that shows what you need.” [Box 8-6]	**Rushing impaired communication:** “This takes too long; forget it.” [Box 8-6]

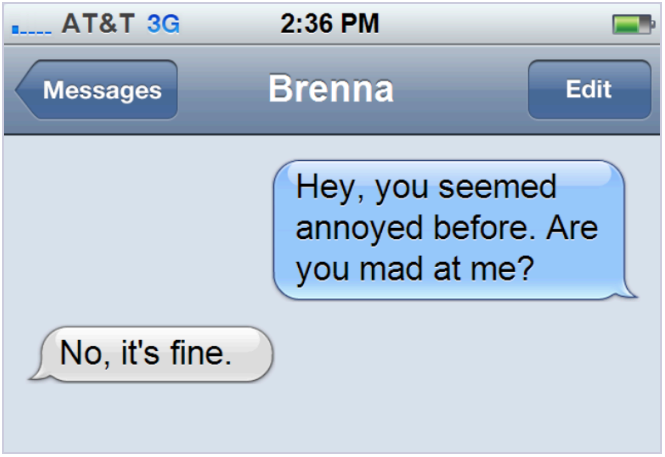
NCLEX test traps

- **Sending ≠ understanding:** Nodding, silence, or receiving instructions is not sufficient verification. Use feedback, qualified interpretation, and teach-back when appropriate. [Slide 5; Activity Slides 24–25]
- **Do not automatically fill silence:** Attentive quiet may permit processing and expression; withdrawal or ignoring the patient is different. [Slide 25; Taylor p. 193]
- **Support is not a guarantee:** “Nothing bad will happen” is false reassurance. Offer presence, truthful information, and realistic help. [Slide 33; Activity Slide 10]
- **Patient-centered is not nurse-centered:** Personal stories, sympathy, advice, approval, and staff defense can derail the patient’s concerns. Empathy and neutral exploration preserve autonomy. [Slides 24, 33]
- **Nonverbal cues are not a diagnosis:** Investigate mismatches without accusing the patient or discarding self-report; account for cultural variation. [Activity Slide 20; Taylor p. 177]
- **Open-ended is not always the only useful format:** Prefer it for feelings and elaboration; neutral focused questions collect specific assessment data. Leading questions suggest a desired answer. Note the instructor’s closed-question classification above. [Slides 25, 33; Activity Slide 17]
- **Language difference is not hearing loss or incapacity:** Do not shout, rely on nodding, or substitute an unqualified interpreter for complex clinical information when qualified help is available. [Activity Slide 25; Box 8-4]
- **Communication does not replace safety assessment:** Hopelessness, thoughts of harm, new confusion, and worsening symptoms require assessment and appropriate escalation—not

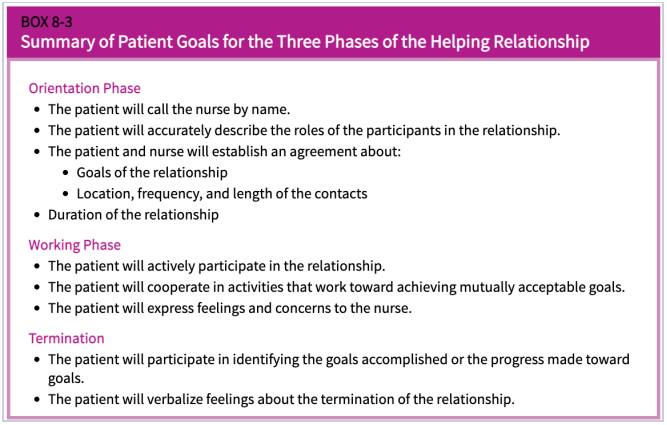
reassurance alone. [Activity Slides 6, 8, 13, 16–17]

- **Professional boundaries persist online:** No patient name, a private account, or deleting a post does not guarantee confidentiality. Assertiveness is also not aggression; address safety concerns without threats or humiliation. [Taylor pp. 178–179] [Taylor pp. 195–196]
- **Adapt without infantilizing:** Speech impairment does not justify assuming incompetence. Allow time, provide usable aids, preserve dignity, and avoid elderspeak. Termination includes shift change—not only discharge. [Taylor p. 188] [Taylor p. 201] [Box 8-6]

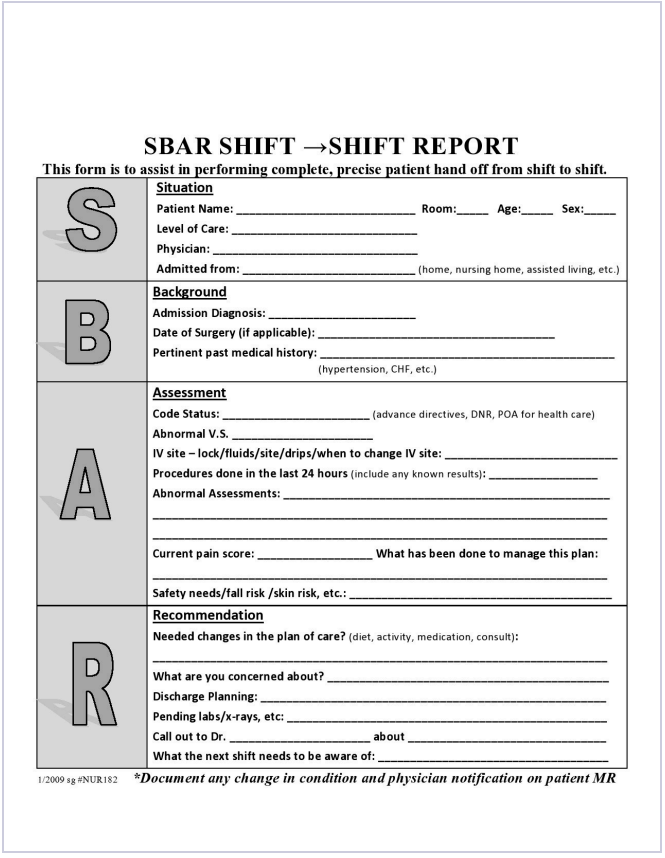
Her slides: pictures



Ch. 8 slide 11



Ch. 8 slide 23



SBAR SHIFT →SHIFT REPORT

This form is to assist in performing complete, precise patient hand off from shift to shift.

S — Situation

Patient Name: _____ Room:_____ Age:_____ Sex:_____
Level of Care: _____
Physician: _____
Admitted from: _____ (home, nursing home, assisted living, etc.)

B — Background

Admission Diagnosis: _____
Date of Surgery (if applicable): _____
Pertinent past medical history: _____
(hypertension, CHF, etc.)

A — Assessment

Code Status: _____ (advance directives, DNR, POA for health care)
Abnormal V.S. _____
IV site – lock/fluids/site/drips/when to change IV site: _____
Procedures done in the last 24 hours (include any known results): _____
Abnormal Assessments: _____
_____
_____
Current pain score: _____ What has been done to manage this plan:
_____
Safety needs/fall risk /skin risk, etc.: _____

R — Recommendation

Needed changes in the plan of care? (diet, activity, medication, consult):
_____
What are you concerned about? _____
Discharge Planning: _____
Pending labs/x-rays, etc: _____
Call out to Dr. _____ about _____
What the next shift needs to be aware of: _____

1/2009 sg #NUR182 **Document any change in condition and physician notification on patient MR*

Ch. 8 slide 45

Lab Skills Weeks 1–5

Week 2 (9/28) — Hand Hygiene & PPE

ATI Skills-Checklists [Page 1]

- Performing Hand Hygiene
- Applying PPE
- Removing PPE

Videos—WATCH [Page 1]

- Gloves
- Transmission-Based Precautions

Skills In Lab [Page 1]

- Hand hygiene
- PPE (don/doff)
- Entering/exiting the room
- Room safety

TracPrac [Page 1]

- Standard precautions with proper hand hygiene
- Don and Doff PPE

Critical (must-pass) points

- Know **when** hand hygiene is required (before/after patient contact, before aseptic tasks, after body-fluid exposure, after touching patient surroundings)
- PPE don/doff sequence must be performed in the correct order without contamination
- Entering/exiting the room: apply PPE **before** entering; remove appropriately **before** leaving
- Room safety: identify hazards, maintain clear pathways, keep call light and belongings within reach
- Standard precautions apply to **all** patients regardless of diagnosis

Week 3 (10/5) — Mobility, Transfers & Body Mechanics

ATI Skills-Checklists [Page 1]

- Moving and Turning Clients in Bed
- Safe and Effective Transfer Techniques
- Transferring Clients

Videos—WATCH [Pages 1–2]

- Effects of Decreased Mobility
- Ergonomic Principles
- Mobility: Ambulation—Crutches
- Mobility: Ambulation—Cane
- Mobility: Ambulation—Nurse Assist
- Mobility: Ambulation—Walker
- Mobility: Transferring—Bed to Stretcher
- Mobility: Transferring—Mechanical Lift

Skills In Lab [Page 1]

- Ambulation / assisted fall

- Wheelchairs
- Positioning / Hoyer lift
- Lateral transfers

TracPrac [Page 1]

- Ambulate with client
- Transfer client to chair, commode, or wheelchair
- Reposition client in bed with safe body mechanics

Critical (must-pass) points

- **Body mechanics:** wide base of support, bend at knees (not back), keep load close, pivot—not twist
- Lock wheelchair brakes before any transfer; use transfer belt when indicated
- Bed → chair transfer: raise bed to appropriate height, place chair close, assist on strong side if applicable
- Hoyer/mechanical lift: verify sling placement and weight capacity **before** lifting
- Repositioning in bed: use draw sheet/turning devices—never drag skin across sheets
- Crutch gait sequence (e.g., swing-to, swing-through) and cane on **strong** side with affected leg stepping first

Week 4 (10/12) — Vital Signs Assessment

ATI Skills-Checklists [Page 2]

- Assessing Body Temperature
- Assessing Peripheral Pulses
- Assessing Respirations
- Assessing Pulse Ox

Videos—WATCH [Page 2]

- Respirations **(2 videos—watch both!)**
- Blood Pressure and Pulse Oximetry
- Vital Signs: Pulse (Radial)
- Vital Signs: Temperature (Axillary)
- Vital Signs: Temperature (Oral)
- Vital Signs: Temperature (Temporal)
- Vital Signs: Temperature (Tympanic)

Skills In Lab [Page 2]

- Electronic vital signs machine: SpO_2, temp, HR
- Manual pulse and respiration

TracPrac [Page 2]

- Pulse Oximetry (intermittent or continuous)
- VS — Blood Pressure — electronic
- VS — Pulse — electronic
- VS — Temperature: Axillary
- VS — Temperature: Oral
- VS — Temperature: Rectal **(discussion only)**
- Manual Pulse
- Manual Respirations
- Complete documentation exercise

Critical (must-pass) points

- **Radial pulse:** palpate 30 sec × 2 (or full 60 sec if irregular); note rate, rhythm, strength
- **Respirations:** count **without** alerting the patient (assess while appearing to take radial pulse); note rate, depth, rhythm, effort
- **Temperature routes:** axillary (~0.5–1 °F lower than oral), oral (no hot/cold liquids 15 min prior), tympanic (pull pinna up-back for adults), temporal
- **Pulse oximetry:** ensure warm perfused digit, no nail polish/artificial nails; SpO_2 < 95 % warrants follow-up
- **Electronic BP:** correct cuff size, arm at heart level, patient seated & relaxed 5 min
- Complete documentation of all vital signs is required for the TracPrac exercise

Week 5 (10/19) — Manual Blood Pressure

ATI Skills-Checklists [Page 3]

- 2-step blood pressure

Videos—WATCH [Page 3]

- 2-step blood pressure

Skills In Lab [Page 3]

- Manual blood pressure: **checklist pt 1** — brachial pulse + PSP (palpated systolic pressure)
- Manual blood pressure: **checklist pt 2** — auscultation

TracPrac [Page 3]

- VS — Blood Pressure — Manual * *(upon successful passing of the manual BP check-off)*

⛔ **MUST-PASS CHECK-OFF** [Page 3]

- **2-Step Manual Blood Pressure Check-Off** — must pass to proceed in TracPrac
- If the student does **not** pass, follow the **remediation protocol** listed in the syllabus
- Instructor note for Week 6 prep: *"Review ATI video and LLUSN Manual BP checklist and practice on as many adult 'patients' as possible"* [Page 3]

Critical (must-pass) points

- **Step 1 — PSP:** palpate brachial pulse; inflate cuff until pulse disappears; deflate slowly; record palpated systolic pressure
- **Step 2 — Auscultation:** place bell/diaphragm over brachial artery; inflate 20–30 mmHg above PSP; deflate 2–3 mmHg/sec
- Identify **Korotkoff sounds**: Phase I (systolic) and Phase V (diastolic / silence)
- Correct cuff size: bladder encircles 80 % of arm
- Patient positioned correctly: back supported, feet flat, arm supported at heart level, no talking

From the assigned reading

The provided source (Weekly Skills & TracPrac Breakdown, Pages 1–3) does not include Taylor textbook excerpts or page/box/table markers. If Taylor reading assignments are distributed separately, verify key points against the ATI skill checklists cited above and the instructor's stated priorities.

NCLEX test traps

- **Hand hygiene:** Wrong answer picks alcohol-based rub when soap & water is required (visible soil, *C. difficile*, before eating, after toileting). Don't confuse the two.

- **PPE order:** Trap answers reverse don/doff sequence. Don Gown→Mask/Goggles→Gloves; Doff Gloves→Goggles→Gown→Mask. Removing the mask first contaminates everything below it.
- **Standard vs. transmission-based:** Standard precautions apply to *every* patient. Answer choices that limit precautions to "known infections only" are wrong.
- **Body mechanics:** Distractors say "lock your knees and bend at the waist" — the correct answer is always **wide stance, bend knees, keep load close**.
- **Temperature:** Trap lists the same number for every route. Remember axillary < oral < rectal/tympanic; temporal is approximate.
- **Pulse oximetry:** Wrong answers ignore factors that **falsely lower** SpO_2 (cold extremities, poor perfusion, nail polish, dark pigmentation) or falsely **reassure** in CO poisoning (SpO_2 reads normal because CO-Hb absorbs at same wavelength).
- **Manual BP:** Trap says inflate cuff "as high as possible" or uses a cuff that's too small (falsely **high** reading) or too large (falsely **low**). Also watch for "deflate rapidly" — correct rate is 2–3 mmHg/sec.
- **Respirations:** Answer that says "tell the patient you are counting respirations" is wrong — patient awareness changes respiratory rate. Count while appearing to assess the radial pulse.
- **Transfer safety:** Trap omits locking wheelchair brakes or uses no transfer belt when needed. **Always** lock brakes and ensure a stable base before moving a patient.
- **Documentation:** Wrong answers skip recording a vital sign or use "WNL" without numeric values. **Record actual measurements** — never leave a VS undocumented.